

ClassicHealth

Insurance Company | Chicago, Illinois | NAIC #58217

MEDICAL • DENTAL • VISION PPO & HMO

2026 Master Plan Reference Guide

Benefits, cost sharing, rules and procedures for all six plans

CH-MED-PPO-2026	ClassicHealth Premier PPO	Medical • PPO
CH-MED-HMO-2026	ClassicHealth Select HMO	Medical • HMO
CH-DEN-PPO-2026	ClassicHealth DentalChoice PPO	Dental • PPO
CH-DEN-HMO-2026	ClassicHealth DentalFirst DHMO	Dental • HMO
CH-VIS-PPO-2026	ClassicHealth VisionPlus PPO	Vision • PPO
CH-VIS-HMO-2026	ClassicHealth VisionCare HMO	Vision • HMO

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1. About ClassicHealth and How to Use This Guide

1.1 Company Profile

ClassicHealth Insurance Company ("ClassicHealth," "the Plan," or "we") is a licensed health, dental and vision carrier domiciled in Illinois and operating in fourteen states. ClassicHealth underwrites and administers fully insured group products and provides administrative services for self-funded employer groups through its wholly owned subsidiary, ClassicHealth Administrators, LLC.

Attribute	Value
Legal entity	ClassicHealth Insurance Company
Domicile	Illinois
NAIC company code	58217
Federal EIN	36-4127905
Year founded	1987
Corporate headquarters	1400 Meridian Tower, 220 North Wacker Drive, Chicago, IL 60606
Lines of business	Group Medical, Group Dental, Group Vision
Product structures offered	PPO (Preferred Provider Organization) and HMO (Health Maintenance Organization)
Licensed states	IL, IN, WI, MI, OH, MO, IA, MN, TX, AZ, CO, NC, GA, FL
Plan year covered by this guide	January 1, 2026 – December 31, 2026
Total covered lives (as of 09/01/2026)	1,142,600
Contracted provider count	486,300 medical; 71,400 dental; 39,800 vision access points
Medicare Part D creditable coverage status	Creditable for all medical plans described in this guide
Grandfathered status under ACA	Not grandfathered — all plans are subject to full ACA requirements
Public website	classichealth.example.com
Member portal	my.classichealth.example.com
Provider portal	providers.classichealth.example.com

1.2 Scope of This Document

This Master Plan Reference Guide is the authoritative consolidated description of all six ClassicHealth plans offered for the 2026 plan year. It consolidates content that is otherwise distributed across individual Summary Plan Descriptions (SPDs), Summaries of Benefits and Coverage (SBCs), Evidence of Coverage (EOC) booklets, the pharmacy formulary, provider manuals, and member service reference cards.

The six plans described are:

Plan ID	Plan marketing name	Product line	Structure	Network
CH-MED-PPO-2026	ClassicHealth Premier PPO	Medical	PPO	ClassicHealth Preferred Network (Tier 1) + Extended Network (Tier 2) + out-of-network

Plan ID	Plan marketing name	Product line	Structure	Network
CH-MED-HMO-2026	ClassicHealth Select HMO	Medical	HMO	ClassicHealth Select Network only
CH-DEN-PPO-2026	ClassicHealth DentalChoice PPO	Dental	PPO	DentalChoice Network + out-of-network
CH-DEN-HMO-2026	ClassicHealth DentalFirst DHMO	Dental	HMO (DHMO)	DentalFirst Network only
CH-VIS-PPO-2026	ClassicHealth VisionPlus PPO	Vision	PPO	VisionPlus Network + out-of-network
CH-VIS-HMO-2026	ClassicHealth VisionCare HMO	Vision	HMO	VisionCare Select Network only

1.3 Order of Precedence

Where documents conflict, the following order of precedence governs, from highest authority to lowest:

1. Applicable federal and state law and binding regulatory guidance.
2. The group policy or contract issued to the employer group, including all riders, endorsements and amendments.
3. The Evidence of Coverage (EOC) or Certificate of Coverage for the specific plan.
4. The Summary Plan Description (SPD).
5. This Master Plan Reference Guide.
6. Summary of Benefits and Coverage (SBC), member ID card text, provider directories, and marketing material.

This guide is a reference and summary document. It does not create contractual rights on its own. Where a benefit described here differs from the governing group policy, the group policy controls. Members should request the EOC for their specific group through Member Services at 1-800-555-0142.

1.4 Document Control

Field	Value
Document ID	CH-MPRG-2026-v1.0
Version	1.0
Effective date	January 1, 2026
Expiration date	December 31, 2026
Owner	Office of Plan Documentation Services, ClassicHealth Insurance Company
Review cycle	Annual, with quarterly formulary supplements (January 1, April 1, July 1, October 1)
Approved by	Chief Medical Officer; VP, Product; General Counsel; Chief Compliance Officer
Language assistance	Available at no cost in Spanish, Polish, Mandarin, Tagalog, Vietnamese, Arabic, Korean, Russian and Hindi
Alternate formats	Large print, braille and audio available on request at 1-800-555-0142

1.5 Key Definitions Used Throughout

A full glossary appears in Section 16. The following six terms are used so frequently that they are defined here.

- **Allowed amount** — the maximum dollar amount the Plan recognizes as payable for a covered service. In-network providers accept the allowed amount as payment in full. Out-of-network providers may bill the member for the difference between their charge and the allowed amount (balance billing), except where prohibited by the No Surprises Act.
- **Deductible** — the amount a member pays for covered services each plan year before the Plan begins paying coinsurance. Copays do not apply to the deductible unless stated otherwise.
- **Coinsurance** — the percentage of the allowed amount the member pays after the deductible is satisfied.
- **Copayment (copay)** — a fixed dollar amount paid at the time of service.
- **Out-of-pocket maximum (OOPM)** — the most a member pays in a plan year for covered in-network services. Once reached, the Plan pays 100% of the allowed amount for covered in-network services for the rest of the plan year.
- **Medically necessary** — health care services that a prudent provider would provide to diagnose or treat an illness, injury, or its symptoms, that are consistent with generally accepted standards of medical practice, clinically appropriate in type, frequency, extent, site and duration, and not primarily for the convenience of the member or provider.

2. Eligibility, Enrollment and Effective Dates

2.1 Employee Eligibility

An employee is eligible to enroll if all of the following are true:

- The employee is classified by the employer as active and regularly scheduled to work at least 30 hours per week (or the hours threshold stated in the group policy, which may not be higher than 40).
- The employee has satisfied the group's waiting period. The ClassicHealth standard waiting period is coverage effective on the **first day of the month following 30 days of continuous employment**. Groups may elect a shorter period, or first-of-month-following-date-of-hire. No group waiting period may exceed 90 days.
- The employee resides or works in the plan's service area. For HMO plans this requirement is strict; see Section 2.6.
- The employee is not covered as a dependent under another ClassicHealth plan sponsored by the same employer.

Seasonal, temporary and variable-hour employees are evaluated using the ACA look-back measurement method when elected by the employer. The standard measurement period is 12 months, the administrative period is 60 days, and the stability period is 12 months.

2.2 Dependent Eligibility

Dependent category	Eligibility rule	Documentation required
Legal spouse	Eligible for all plans	Marriage certificate
Domestic partner	Eligible where the group has elected the domestic partner rider	Signed Affidavit of Domestic Partnership (form CH-DP-100) or state registration
Natural or adopted child	Eligible to the end of the month in which the child turns 26	Birth certificate or adoption placement order
Stepchild	Eligible to age 26 while the marriage to the child's parent remains in force	Birth certificate plus marriage certificate
Child under legal guardianship	Eligible to age 26	Court order of guardianship
Child subject to a QMCSO or NMSN	Enrolled as ordered, regardless of open enrollment timing	Qualified Medical Child Support Order or National Medical Support Notice
Disabled dependent over age 26	Eligible without age limit if incapable of self-sustaining employment due to a disability that began before age 26 and the member is chiefly dependent on the employee for support	Form CH-DD-220 plus physician certification, submitted within 31 days of the child reaching 26 and recertified every 24 months thereafter
Grandchild	Not eligible unless the employee has legal guardianship or the grandchild is a tax dependent placed by court order	Court order

Coverage for a dependent child ends at 11:59 p.m. on the last day of the month in which the child turns 26, except for a certified disabled dependent.

2.3 Open Enrollment

The annual open enrollment period for the 2026 plan year ran from **November 1, 2025 through November 30, 2025**, with elections effective January 1, 2026. Outside of open enrollment, an employee may enroll, drop or change plans only in connection with a qualifying life event.

2.4 Qualifying Life Events and Special Enrollment

A member has **30 days** from the date of the event to request a change, except where noted.

Qualifying life event	Change permitted	Notice window	Coverage effective date
Marriage	Add spouse and stepchildren; change plan tier	30 days	First of the month following the request
Birth	Add newborn	30 days	Date of birth
Adoption or placement for adoption	Add child	30 days	Date of placement
Divorce or legal separation	Drop spouse; COBRA offered to the ex-spouse	30 days (COBRA notice within 60 days)	End of the month of the decree
Death of a covered dependent	Drop the dependent	30 days	Date of death
Loss of other group or individual coverage	Enroll self and dependents	30 days	Day following the loss of prior coverage
Gain of other coverage	Drop ClassicHealth coverage	30 days	End of the month
Spouse's employer open enrollment	Add or drop	30 days	Effective date of the spouse's plan
Loss of Medicaid or CHIP eligibility	Enroll	60 days	Day following the loss
Gain of Medicaid or CHIP premium assistance eligibility	Enroll	60 days	First of the month following the request
Permanent relocation outside the HMO service area	Transfer to the PPO plan	30 days	First of the month following the move
Change in employment status affecting eligibility	Enroll or terminate	30 days	Per the group policy
Court order (QMCSO / NMSN)	Enroll the named child	As ordered	As ordered, retroactive if required
Entitlement to Medicare	Drop or change coverage	30 days	First of the month of entitlement

A newborn is covered automatically for the first 31 days from birth. To continue coverage past 31 days, the employee must formally enroll the newborn within 30 days of birth and pay any additional premium. Failure to enroll within the window means the newborn has no coverage after day 31 until the next open enrollment.

2.5 Termination of Coverage

Coverage ends on the earliest of:

- The last day of the calendar month in which employment ends or the employee ceases to meet the eligibility rules.
- The last day of the period for which premium was paid, if premium is not paid within the 31-day grace period.
- The date the group policy terminates.
- The date the member is no longer an eligible dependent.

- The date of the member's death (dependent coverage continues to the end of that month, then COBRA is offered).
- The date the member commits fraud or an intentional misrepresentation of material fact. Rescission is retroactive and requires 30 days' advance written notice.

Continuation options are described in Section 14.

2.6 Service Area Rules

PPO plans may be issued anywhere ClassicHealth is licensed. HMO plans require the subscriber's permanent residence or primary work location to be within an approved HMO service area county.

State	HMO service area (counties / metros)
Illinois	Cook, DuPage, Lake, Will, Kane, McHenry, Winnebago, Sangamon, St. Clair, Madison
Indiana	Lake, Porter, Marion, Hamilton, Allen
Wisconsin	Milwaukee, Waukesha, Dane, Racine, Kenosha
Michigan	Wayne, Oakland, Macomb, Kent, Washtenaw
Ohio	Cuyahoga, Franklin, Hamilton, Summit, Montgomery
Missouri	St. Louis, St. Louis City, St. Charles, Jackson, Clay
Iowa	Polk, Dallas, Linn, Scott, Johnson
Minnesota	Hennepin, Ramsey, Dakota, Anoka, Washington
Texas	Harris, Dallas, Tarrant, Bexar, Travis, Collin, Denton
Arizona	Maricopa, Pima, Pinal
Colorado	Denver, Jefferson, Arapahoe, Adams, Douglas, Boulder, El Paso
North Carolina	Mecklenburg, Wake, Guilford, Durham, Forsyth
Georgia	Fulton, DeKalb, Cobb, Gwinnett, Clayton, Chatham
Florida	Miami-Dade, Broward, Palm Beach, Orange, Hillsborough, Duval, Pinellas

A member who moves permanently outside the HMO service area must transfer to the Premier PPO within 30 days. Members temporarily outside the service area (students, extended travel) retain emergency and urgent care coverage only; see Section 3.9.

2.7 Member Identification Cards

A single ID card is issued per member showing all elected coverages. Card fields and their meaning:

Field on card	Example	Meaning
Member ID	CH8840173201	Unique 12-character subscriber identifier; dependents share the base ID with a 2-digit suffix
Group number	GRP-40219	Employer group contract number
Plan code	MED-PPO / MED-HMO	Identifies the medical product
RxBIN	610987	Pharmacy routing number
RxPCN	CHRX2026	Pharmacy processor control number
RxGRP	CHPPO01 or CHHMO01	Pharmacy group

Field on card	Example	Meaning
PCP name	Printed on HMO cards only	Assigned primary care physician
Copay strip	OV \$30 / SPC \$50 / ER \$350	Quick-reference cost shares

3. ClassicHealth Premier PPO — Medical (CH-MED-PPO-2026)

3.1 Plan Design at a Glance

The Premier PPO is an open-access preferred provider plan. Members may see any licensed provider without a referral. Cost sharing is lowest in Tier 1, higher in Tier 2, and highest out of network.

Provision	Tier 1 — Preferred Network	Tier 2 — Extended Network	Out-of-Network
Annual deductible — individual	\$1,500	\$2,250	\$3,000
Annual deductible — family	\$3,000	\$4,500	\$6,000
Deductible structure	Embedded	Embedded	Embedded
Plan coinsurance after deductible	Plan pays 80%	Plan pays 70%	Plan pays 50% of allowed amount
Out-of-pocket maximum — individual	\$5,000	\$7,000	\$10,000
Out-of-pocket maximum — family	\$10,000	\$14,000	\$20,000
Referral required	No	No	No
PCP selection required	No (recommended)	No	No
Lifetime or annual dollar maximum on essential health benefits	None	None	None
Balance billing possible	No	No	Yes, except where the No Surprises Act applies

The family deductible is embedded. No single member within a family contract pays more than the individual deductible (\$1,500 Tier 1) or more than the individual out-of-pocket maximum (\$5,000 Tier 1) before the Plan begins paying for that member's covered services at 100%. The family amount is satisfied by any combination of family members' expenses.

Amounts that count toward the out-of-pocket maximum: deductibles, coinsurance, copayments, and prescription drug cost sharing. Amounts that do **not** count: premium contributions, balance-billed charges from out-of-network providers, penalties for failure to obtain prior authorization, charges for non-covered services, and amounts above the plan's frequency limits.

3.2 Medical Benefit Grid — Premier PPO

Table 3-A. Premier PPO cost sharing by service category. "Ded." = deductible.

Service	Tier 1 Preferred	Tier 2 Extended	Out-of-Network
Preventive care (ACA-required)	\$0, ded. waived	\$0, ded. waived	50% after ded.
Primary care office visit	\$30 copay, ded. waived	\$50 copay, ded. waived	50% after ded.
Specialist office visit	\$50 copay, ded. waived	\$75 copay, ded. waived	50% after ded.

Service	Tier 1 Preferred	Tier 2 Extended	Out-of-Network
Virtual primary care (ClassicHealth Virtual Care)	\$0	\$0	Not covered
Virtual urgent care	\$15 copay	\$15 copay	Not covered
Retail clinic / convenience care	\$30 copay	\$50 copay	50% after ded.
Urgent care center	\$75 copay, ded. waived	\$75 copay, ded. waived	\$75 copay, ded. waived
Emergency room	\$350 copay then 20% after ded.	Paid at Tier 1	Paid at Tier 1
Ambulance — ground	20% after ded.	20% after ded.	20% after ded.
Ambulance — air	20% after ded.	20% after ded.	20% after ded.
Inpatient hospital — facility	20% after ded.	30% after ded.	50% after ded., PA required
Inpatient professional services	20% after ded.	30% after ded.	50% after ded.
Outpatient surgery — ambulatory surgery center	20% after ded.	30% after ded.	50% after ded.
Outpatient surgery — hospital based	20% after ded.	30% after ded.	50% after ded.
Diagnostic lab — freestanding	\$0, ded. waived	30% after ded.	50% after ded.
Diagnostic lab — hospital outpatient	20% after ded.	30% after ded.	50% after ded.
Basic X-ray	\$0, ded. waived	30% after ded.	50% after ded.
Advanced imaging (MRI, CT, PET, nuclear)	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required
Maternity — prenatal and postpartum care	\$0, ded. waived	\$0, ded. waived	50% after ded.
Maternity — delivery and facility	20% after ded.	30% after ded.	50% after ded.
Newborn nursery care	20% after ded.	30% after ded.	50% after ded.
Physical, occupational, speech therapy	\$50 copay	\$75 copay	50% after ded.
Chiropractic care	\$50 copay	\$75 copay	50% after ded.
Acupuncture	\$50 copay	Not covered	Not covered
Durable medical equipment	20% after ded.	30% after ded.	50% after ded.
Prosthetics and orthotics	20% after ded.	30% after ded.	50% after ded.
Home health care	20% after ded.	30% after ded.	50% after ded., PA required
Skilled nursing facility	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required
Hospice care	\$0	\$0	50% after ded.
Mental health — outpatient office	\$30 copay, ded. waived	\$50 copay, ded. waived	50% after ded.
Mental health — intensive outpatient / partial hospitalization	20% after ded.	30% after ded.	50% after ded., PA required
Mental health — inpatient	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required

Service	Tier 1 Preferred	Tier 2 Extended	Out-of-Network
Substance use disorder — all levels	Same as mental health	Same as mental health	Same as mental health
Applied behavior analysis (autism)	\$30 copay, PA required	\$50 copay, PA required	50% after ded., PA required
Allergy testing and injections	\$0 with an office visit copay	30% after ded.	50% after ded.
Dialysis	20% after ded.	20% after ded.	50% after ded., PA required
Infusion therapy — home or ambulatory	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required
Transplant services (Centers of Excellence)	20% after ded., PA required	Must use Center of Excellence	Not covered except emergency stabilization
Bariatric surgery	20% after ded., PA and criteria required	30% after ded., PA required	Not covered
Hearing exam	\$50 copay	\$75 copay	50% after ded.
Hearing aids	20% after ded., \$2,500 per ear every 36 months	30% after ded., same limit	50% after ded., same limit
Gender-affirming care	Covered at the applicable service cost share, PA required	Same	50% after ded., PA required
Fertility services — diagnosis and testing	20% after ded.	30% after ded.	50% after ded.
Fertility services — IVF and ART	20% after ded., \$15,000 lifetime maximum, PA required	Not covered	Not covered
Clinical trial routine patient costs	Covered at the applicable service cost share	Same	50% after ded.
Genetic testing and counseling	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required
Sleep study — home based	\$0, ded. waived	30% after ded.	50% after ded.
Sleep study — attended in facility	20% after ded., PA required	30% after ded., PA required	50% after ded., PA required
Diabetes education and self-management training	\$0, ded. waived	\$0, ded. waived	50% after ded.
Diabetic supplies (meters, strips, lancets)	\$0, ded. waived	\$0, ded. waived	50% after ded.

3.3 Visit and Day Limits — Premier PPO

Benefit	Limit per plan year
Physical, occupational and speech therapy combined	30 visits, in-network and out-of-network combined
Chiropractic manipulation	20 visits
Acupuncture	12 visits
Skilled nursing facility	60 days per plan year
Home health care	100 visits per plan year

Benefit	Limit per plan year
Inpatient rehabilitation	45 days per plan year
Hospice — inpatient respite	5 consecutive days per occurrence
Cardiac rehabilitation	36 sessions per event
Pulmonary rehabilitation	24 sessions per plan year
Applied behavior analysis	No visit limit; medical necessity review every 6 months
Routine eye exam under the medical plan	Not covered — see the vision plan
Nutritional counseling	6 visits (unlimited for diabetes)
Smoking cessation counseling	8 sessions per plan year at \$0

3.4 Preventive Services Covered at 100%

Covered at \$0 with no deductible when received in Tier 1 or Tier 2. Frequency follows USPSTF grade A and B recommendations, ACIP immunization schedules, HRSA women's preventive guidelines and Bright Futures for children.

- Annual wellness exam and routine physical, one per plan year for adults; per the Bright Futures schedule for children.
- Immunizations recommended by ACIP, including influenza, COVID-19, RSV, shingles, pneumococcal, HPV, Tdap and childhood series.
- Screening colonoscopy beginning at age 45, once every 10 years; follow-up colonoscopy after a positive non-invasive stool test is also covered at \$0. Polyp removal during a screening colonoscopy remains a preventive service.
- Mammography, one baseline at 35–39 and annually beginning at age 40; 3D tomosynthesis included.
- Cervical cancer screening per guideline, including HPV co-testing.
- Prostate cancer screening (PSA) beginning at age 50, or 45 with elevated risk.
- Lung cancer screening by low-dose CT for adults 50–80 with a 20 pack-year history.
- Bone density screening for women 65+ and younger women with risk factors.
- Lipid, blood pressure, diabetes and obesity screening.
- Depression, anxiety, and unhealthy alcohol and drug use screening.
- Sexually transmitted infection screening and PrEP for HIV, including required office visits and labs.
- All FDA-approved contraceptive methods for women, including sterilization, at \$0. Vasectomy is covered at 20% after deductible.
- Breastfeeding support, counseling and one double-electric breast pump per pregnancy.
- Prenatal screenings and postpartum depression screening.

If a preventive visit results in the evaluation or treatment of a new or existing problem, the provider may bill a separate diagnostic office visit. The diagnostic portion is subject to the regular office visit copay. This is a common source of member billing questions; see FAQ items 12 and 13.

3.5 Emergency and Urgent Care

Emergency services are always paid at the Tier 1 level regardless of where they are received, in the United States or abroad. The Plan applies the prudent layperson standard: coverage is based on the member's presenting symptoms, not on the final diagnosis.

- The \$350 emergency room copay is waived if the member is admitted as an inpatient directly from the emergency department. Inpatient cost sharing then applies.
- Prior authorization is never required for emergency services.
- The Plan must be notified within 48 hours, or as soon as reasonably possible, of an emergency admission to an out-of-network facility so that care coordination and transfer planning can begin.
- Post-stabilization care at an out-of-network facility is covered at the Tier 1 level until the member can be safely transferred, or until the member gives written consent to continued out-of-network care under the No Surprises Act notice-and-consent process.
- Urgent care is covered at the same \$75 copay in and out of network.
- Care received outside the United States is covered for emergency and urgent conditions only. Members pay the provider and submit an international claim on form CH-INTL-500 within 12 months, with an itemized bill and proof of payment; reimbursement is made in U.S. dollars at the exchange rate on the date of service.

4. ClassicHealth Select HMO — Medical (CH-MED-HMO-2026)

4.1 Plan Design at a Glance

The Select HMO is a coordinated-care plan built on a narrower network with lower cost sharing. Every member must select a primary care physician who coordinates all care and issues referrals to specialists.

Provision	In-Network (Select Network)	Out-of-Network
Annual deductible — individual	\$500	Not applicable — no coverage
Annual deductible — family	\$1,000	Not applicable
Deductible structure	Embedded	—
Out-of-pocket maximum — individual	\$3,500	Not applicable
Out-of-pocket maximum — family	\$7,000	Not applicable
PCP selection required	Yes	—
Specialist referral required	Yes, except for the direct-access services listed in Section 4.4	—
Out-of-network coverage	Emergency and urgent care only; also covered for authorized out-of-network care when no in-network provider is available	—
Lifetime or annual dollar maximum	None	—

4.2 Medical Benefit Grid — Select HMO

Table 4-A. Select HMO cost sharing. All benefits require use of the Select Network unless otherwise stated.

Service	Member cost share
Preventive care (ACA-required)	\$0, deductible waived
Primary care office visit	\$20 copay, deductible waived
Specialist office visit	\$40 copay, deductible waived, referral required
Virtual primary care	\$0
Virtual urgent care	\$10 copay
Retail clinic	\$20 copay
Urgent care center	\$50 copay, deductible waived (covered in and out of network)
Emergency room	\$300 copay, deductible waived, waived if admitted
Ambulance — ground and air	\$150 copay per transport
Inpatient hospital — facility	\$750 copay per admission after deductible, prior authorization required
Inpatient professional services	\$0 after the facility copay
Outpatient surgery — ambulatory surgery center	\$300 copay after deductible

Service	Member cost share
Outpatient surgery — hospital based	\$500 copay after deductible
Diagnostic lab	\$0, deductible waived
Basic X-ray	\$25 copay
Advanced imaging (MRI, CT, PET, nuclear)	\$150 copay after deductible, prior authorization required
Maternity — prenatal and postpartum	\$0
Maternity — delivery and facility	\$750 copay per admission after deductible
Newborn nursery care	\$0 when the mother's admission copay has been applied
Physical, occupational, speech therapy	\$30 copay per visit
Chiropractic care	\$30 copay per visit
Acupuncture	Not covered
Durable medical equipment	20% after deductible, prior authorization over \$1,000
Prosthetics and orthotics	20% after deductible
Home health care	\$0 after deductible, prior authorization required
Skilled nursing facility	\$250 copay per admission after deductible, prior authorization required
Hospice care	\$0
Mental health — outpatient office	\$20 copay, deductible waived
Mental health — intensive outpatient / partial hospitalization	\$50 copay per day
Mental health — inpatient	\$750 copay per admission after deductible, prior authorization required
Substance use disorder — all levels	Same as mental health
Applied behavior analysis (autism)	\$20 copay, prior authorization required
Allergy testing and injections	\$0 with an office visit copay
Dialysis	\$0 after deductible
Infusion therapy	\$0 after deductible, prior authorization required
Transplant services	\$750 copay per admission, Center of Excellence required, prior authorization required
Bariatric surgery	\$750 copay per admission, prior authorization and clinical criteria required
Hearing exam	\$40 copay
Hearing aids	20% after deductible, \$2,500 per ear every 36 months
Gender-affirming care	Applicable service cost share, prior authorization required
Fertility services — diagnosis and testing	\$40 copay
Fertility services — IVF and ART	Not covered
Genetic testing and counseling	\$0 after deductible, prior authorization required
Sleep study — home based	\$0
Sleep study — attended in facility	\$150 copay after deductible, prior authorization required

Service	Member cost share
Diabetes education and supplies	\$0
Clinical trial routine patient costs	Applicable service cost share

4.3 Visit and Day Limits — Select HMO

Benefit	Limit per plan year
Physical, occupational and speech therapy combined	25 visits
Chiropractic manipulation	12 visits
Skilled nursing facility	45 days
Home health care	90 visits
Inpatient rehabilitation	30 days
Cardiac rehabilitation	36 sessions per event
Pulmonary rehabilitation	24 sessions
Nutritional counseling	6 visits (unlimited for diabetes)
Smoking cessation counseling	8 sessions at \$0

4.4 Primary Care Physicians and Referrals

Each covered member must designate a PCP. If a member does not choose one within 30 days of the effective date, ClassicHealth assigns a PCP based on geographic proximity and the member's claims history; the member may change the assignment at any time.

- Eligible PCP specialties: family medicine, internal medicine, pediatrics, general practice, and obstetrics-gynecology when the physician has agreed to serve as a PCP.
- A PCP change requested by the 20th of a month is effective the first day of the following month. Changes requested after the 20th are effective the first day of the second following month.
- Referrals are issued electronically by the PCP and are visible in the member portal. A standard referral authorizes up to **6 visits within 90 days** to the named specialist. Extended referrals of up to 12 months are available for chronic conditions.
- A referral is not a guarantee of payment. The service must still be covered, the member must still be eligible on the date of service, and any separately required prior authorization must still be obtained.

Direct access — no referral required for: obstetric and gynecologic care from an in-network OB-GYN; behavioral health and substance use services from any in-network behavioral provider; annual routine eye exams under the vision plan; emergency and urgent care; family planning and contraceptive services; and treatment for sexually transmitted infections.

4.5 Continuity and Transition of Care

A new member in active treatment may request continued care from a provider who is not in the Select Network for a transition period.

Situation	Transition period allowed
Newly enrolled member in an active course of treatment	90 days from the effective date

Situation	Transition period allowed
Second or third trimester of pregnancy at enrollment	Through delivery and postpartum care
Terminal illness with a life expectancy under 12 months	For the duration of the illness
In-network provider leaves the network mid-treatment (not for cause)	90 days from the termination notice
Scheduled non-elective surgery within 60 days of the effective date	Through the surgery and follow-up
Newborn or newly adopted child under 36 months	12 months

Requests are submitted on form CH-TOC-310. Approved transition care is paid at the in-network benefit level, and the non-participating provider must accept the ClassicHealth allowed amount as payment in full.

5. Prescription Drug Benefit

5.1 Program Structure

The pharmacy benefit is integrated with each medical plan and administered by ClassicHealth Pharmacy Services with Meridian Specialty Rx as the exclusive specialty pharmacy. The retail network includes 67,000 pharmacies nationwide.

Provision	Premier PPO	Select HMO
Separate prescription drug deductible	\$150 per member, applies to Tier 3 and Tier 4 only	None
Prescription cost sharing counts toward the medical out-of-pocket maximum	Yes	Yes
Formulary	ClassicHealth Advantage Formulary, 4 tiers	ClassicHealth Select Formulary, 4 tiers
Retail day supply	Up to 30 days; up to 90 days at a Preferred90 pharmacy	Up to 30 days; up to 90 days at a Preferred90 pharmacy
Mail order day supply	Up to 90 days	Up to 90 days
Specialty day supply	30 days only	30 days only
Out-of-network pharmacy	Covered at 50% of the allowed amount after the applicable copay; emergency only	Not covered except emergency

5.2 Cost Sharing by Tier

Table 5-A. Prescription drug cost sharing. Mail order and Preferred90 amounts are for a 90-day supply.

Tier	Description	PPO retail 30 days	PPO mail 90 days	HMO retail 30 days	HMO mail 90 days
Tier 1	Preferred generic	\$10	\$25	\$5	\$12.50
Tier 2	Preferred brand	\$40	\$100	\$30	\$75
Tier 3	Non-preferred brand and non-preferred generic	\$75 after Rx deductible	\$187.50 after Rx deductible	\$60	\$150
Tier 4	Specialty (brand or generic)	25% coinsurance, \$250 maximum per fill, after Rx deductible	Not available — 30-day fills only	20% coinsurance, \$200 maximum per fill	Not available

Additional pharmacy rules:

- **Insulin cap.** All insulin products on the formulary are capped at **\$35 per 30-day supply** on both plans, regardless of tier, and the pharmacy deductible does not apply.
- **ACA preventive drugs at \$0.** Generic contraceptives, generic statins for eligible adults 40–75, tobacco cessation products, folic acid for women of childbearing age, low-dose aspirin where indicated, bowel preparation kits for screening colonoscopy, and all ACIP-recommended vaccines dispensed at a network pharmacy.
- **Generic substitution.** If a member requests a brand-name drug when an A-rated generic equivalent exists, the member pays the Tier 1 copay plus the full difference in cost between the brand and the generic. The difference

does not count toward the out-of-pocket maximum.

- **Dispense as written.** If the prescriber indicates DAW-1 and the Plan approves a medical necessity exception, the brand is covered at the Tier 2 copay.
- **Vacation and lost-medication overrides.** One vacation supply override per drug per plan year, up to a 90-day supply; one lost-or-stolen override per drug per plan year with a police report for controlled substances.
- **Partial fill.** Controlled substances may be partially filled at the member's request with no additional copay for the balance of the original quantity.

5.3 Utilization Management in Pharmacy

Control	What it means	Typical drug classes affected
Prior authorization (PA)	Clinical review before the drug is covered	Specialty biologics, GLP-1 agonists for weight management, growth hormone, hepatitis C agents, PCSK9 inhibitors, testosterone, compounded drugs over \$300
Step therapy (ST)	A preferred first-line agent must be tried first, or documented as contraindicated or previously failed	PPIs, SNRIs and SSRIs, DMARDs, non-preferred inhalers, topical steroids
Quantity limits (QL)	FDA-labeled or guideline-based maximum quantity per fill or per period	Triptans, opioids, sedative-hypnotics, PDE5 inhibitors, nasal steroids
Age limits	Coverage restricted to a labeled age range	Isotretinoin, ADHD stimulants, testosterone
Specialty exclusive pharmacy	Must fill through Meridian Specialty Rx	All Tier 4 drugs
Site of care redirection	Provider-administered specialty drugs move from hospital outpatient to home or ambulatory infusion when clinically appropriate	Infliximab, immunoglobulin, rituximab

Pharmacy prior authorization decisions are made within **72 hours** for standard requests and **24 hours** for expedited requests. If the Plan does not respond within the required timeframe, the request is deemed approved.

5.4 Formulary Governance

The Pharmacy and Therapeutics Committee — 14 voting members including 9 practicing physicians, 3 pharmacists, and 2 non-voting plan staff — meets quarterly. Formulary updates are published on January 1, April 1, July 1 and October 1.

- Members receive **60 days' advance written notice** before a drug is removed from the formulary or moved to a higher tier mid-year, unless the change is due to an FDA safety withdrawal or the release of a generic equivalent.
- A member currently taking a drug that is removed mid-year may continue at the prior cost share through the end of the plan year by requesting continuity of therapy.
- A formulary exception may be requested by the prescriber. Standard exceptions are decided within 72 hours; expedited exceptions within 24 hours. If granted, the drug is covered at the Tier 3 cost share (Tier 4 rules apply to specialty drugs) for the duration of the prescription including refills.
- If a formulary exception is denied, the member may request an independent external review, which must be decided within 72 hours for standard and 24 hours for expedited requests.

6. ClassicHealth DentalChoice PPO (CH-DEN-PPO-2026)

6.1 Plan Design at a Glance

DentalChoice PPO allows a member to see any licensed dentist. Using a DentalChoice network dentist lowers cost sharing and protects the member from balance billing, because network dentists accept the ClassicHealth contracted fee as payment in full.

Provision	In-Network	Out-of-Network
Annual deductible — individual	\$50	\$75
Annual deductible — family	\$150	\$225
Deductible waived for	Class I diagnostic and preventive services	Class I diagnostic and preventive services
Annual benefit maximum per member	\$2,000 (combined in- and out-of-network)	\$2,000 (combined)
Orthodontia lifetime maximum	\$2,000 per eligible dependent child	\$2,000
Basis of payment	Contracted fee schedule	90th percentile of usual, customary and reasonable charges for the ZIP code
Balance billing	Not permitted	Permitted
Referral required	No	No
Preventive services counted against the annual maximum	No — Class I is exempt from the annual maximum in network	Yes

6.2 Coverage by Class

Table 6-A. DentalChoice PPO coverage levels. Percentages are the share the Plan pays of the allowed amount.

Class	Services included	Plan pays in-network	Plan pays out-of-network	Deductible applies	Waiting period
Class I — Diagnostic and Preventive	Oral evaluations, prophylaxis (cleaning), bitewing and full-mouth radiographs, fluoride, sealants, space maintainers, palliative emergency treatment	100%	80%	No	None
Class II — Basic Restorative	Amalgam and composite fillings, simple extractions, surgical extractions, root canal therapy (endodontics), periodontal scaling and root planing, periodontal maintenance, denture repair and reline, stainless steel crowns, general anesthesia when medically necessary	80%	60%	Yes	None
Class III — Major Restorative	Crowns, inlays and onlays, bridges (fixed partial dentures), complete and partial dentures, implants and implant-supported prosthetics, periodontal surgery (osseous surgery, gingivectomy), occlusal guards	50%	40%	Yes	12 months for members who enroll after their first eligibility date

Class	Services included	Plan pays in-network	Plan pays out-of-network	Deductible applies	Waiting period
Class IV — Orthodontia	Comprehensive orthodontic treatment, appliances, adjustments and retention for eligible dependent children under age 19 at the start of treatment	50% to the \$2,000 lifetime maximum	50% to the \$2,000 lifetime maximum	No	12 months

6.3 Frequency and Age Limitations

Service	Limitation
Periodic oral evaluation	2 per plan year
Comprehensive oral evaluation	1 per dentist every 36 months
Prophylaxis (routine cleaning)	2 per plan year; a third and fourth cleaning are covered at 100% for members who are pregnant, have diabetes, have documented periodontal disease, or have had a heart valve replacement (Enhanced Benefit — enroll by calling 1-800-555-0163)
Bitewing radiographs	1 set per plan year
Full-mouth series or panoramic film	1 every 60 months
Periapical radiographs	As needed for diagnosis
Topical fluoride	2 per plan year to age 19
Sealants	Permanent molars only, to age 16, 1 per tooth every 36 months
Space maintainers	To age 14, for premature loss of primary teeth
Periodontal scaling and root planing	1 per quadrant every 24 months
Periodontal maintenance	4 per plan year, alternating with prophylaxis; total cleanings of all types not to exceed 4 per year
Crowns, inlays, onlays	1 per tooth every 60 months
Bridges and dentures	1 every 60 months per arch or per span
Denture reline or rebase	1 every 36 months, no sooner than 6 months after placement
Implants	1 per missing tooth site per lifetime, subject to the Class III percentage and the annual maximum
Occlusal guard for bruxism	1 every 36 months
Root canal retreatment	Covered once per tooth, no sooner than 24 months after the original therapy
General anesthesia or IV sedation	Covered only with an eligible surgical procedure, or for members under age 7 or with a documented developmental disability

6.4 Provisions Members Ask About Most

- **Missing tooth clause.** Replacement of a tooth that was extracted before the member's coverage effective date is covered only after the member has been continuously enrolled for 12 months. The clause does not apply to teeth extracted while covered by the Plan.
- **Alternate benefit provision (least expensive professionally acceptable treatment).** When more than one course of treatment can correct a condition, the Plan pays toward the least expensive professionally acceptable alternative. Example: if a composite (tooth-colored) filling is placed on a posterior tooth, the Plan pays the amalgam allowance and the member pays the difference. The member and dentist may still choose the more expensive treatment.
- **Pre-treatment estimate.** Strongly recommended for any treatment plan expected to exceed **\$500**. The dentist submits the proposed treatment on an ADA claim form marked "pre-treatment estimate." ClassicHealth returns a written estimate within 10 business days showing what the Plan will pay and what the member will owe. An estimate is valid for 90 days and is not a guarantee of payment.
- **Work in progress.** A crown, bridge or denture is considered incurred on the date the tooth is prepared or the final impression is taken, not the seat date. Root canal therapy is incurred on the date the canal is opened. Orthodontia is incurred on the banding date.
- **Coordination with the medical plan.** Treatment of accidental injury to sound natural teeth, oral surgery for tumors and cysts, temporomandibular joint (TMJ) disorder, and dental care required as a direct result of cancer treatment are covered under the medical plan first. The dental plan pays as secondary.
- **Orthodontia payment schedule.** ClassicHealth pays 25% of the approved case fee at banding and the remainder in equal quarterly installments over the treatment period, not to exceed the \$2,000 lifetime maximum. If coverage ends mid-treatment, payments stop; there is no lump-sum settlement.

6.5 Dental Exclusions

The following are not covered under either dental plan:

- Services that are cosmetic in nature, including veneers, tooth whitening, and personalization or characterization of dentures.
- Replacement of lost, stolen or missing appliances or prosthetics.
- Duplicate or spare appliances and prosthetics.
- Orthodontia for adults age 19 and over under the PPO (the DHMO does offer an adult orthodontic copay — see Section 7).
- Services for which no charge is made, or which are provided at no cost by a government agency.
- Experimental or investigational procedures and techniques.
- Charges for broken appointments, completion of claim forms, or copies of records.
- Athletic mouthguards.
- Myofunctional therapy, and treatment of jaw fractures or dislocations (covered by the medical plan).
- Precision attachments, stress breakers, and overdentures with copings.
- Instruction in plaque control, oral hygiene or diet, when billed separately.
- Any service started before the effective date of coverage or completed after coverage ends, except as described under work in progress.

7. ClassicHealth DentalFirst DHMO (CH-DEN-HMO-2026)

7.1 Plan Design at a Glance

DentalFirst is a prepaid dental plan built on a fixed copayment schedule. There are no deductibles, no annual maximums, no claim forms and no waiting periods. Members must receive all care from their assigned Participating General Dentist (PGD).

Provision	Value
Annual deductible	None
Annual benefit maximum	None
Waiting periods	None
Claim forms	None — the PGD is paid directly by capitation and by the member's copay
Participating General Dentist required	Yes; selected at enrollment
Specialist care	By PGD referral to a DentalFirst network specialist only
Out-of-network coverage	None, except out-of-area emergency palliative treatment reimbursed up to \$75 per incident
Pre-existing conditions excluded	No — treatment in progress at enrollment is covered subject to the copay schedule

Members may change their PGD by calling 1-800-555-0163 or through the member portal. A change requested by the 20th of the month is effective the first of the following month. A member with a course of treatment in progress must complete it or make arrangements with the current PGD before transferring.

7.2 Representative Copayment Schedule

Table 7-A. DentalFirst DHMO member copayments. This is a representative extract; the complete schedule (form CH-DHMO-SCHED-2026) lists 420 ADA procedure codes.

ADA code	Procedure	Member copay
D0120	Periodic oral evaluation	\$0
D0150	Comprehensive oral evaluation	\$0
D0210	Intraoral complete series of radiographic images	\$0
D0274	Bitewings, four films	\$0
D0330	Panoramic radiographic image	\$0
D1110	Prophylaxis, adult (2 per year)	\$0
D1110	Prophylaxis, adult, each additional	\$45
D1120	Prophylaxis, child (2 per year)	\$0
D1206	Topical fluoride varnish	\$0
D1351	Sealant, per tooth	\$12
D1516	Space maintainer, fixed, bilateral	\$95
D2140	Amalgam, one surface, primary or permanent	\$18

ADA code	Procedure	Member copay
D2150	Amalgam, two surfaces	\$24
D2330	Resin-based composite, one surface, anterior	\$30
D2391	Resin-based composite, one surface, posterior	\$48
D2740	Crown, porcelain or ceramic	\$425
D2750	Crown, porcelain fused to high noble metal	\$395
D2950	Core buildup, including any pins	\$75
D2954	Prefabricated post and core	\$85
D3220	Therapeutic pulpotomy	\$60
D3310	Endodontic therapy, anterior tooth	\$215
D3320	Endodontic therapy, premolar	\$265
D3330	Endodontic therapy, molar	\$325
D4341	Periodontal scaling and root planing, per quadrant	\$85
D4260	Osseous surgery, per quadrant	\$340
D4910	Periodontal maintenance	\$45
D5110	Complete denture, maxillary	\$450
D5213	Maxillary partial denture, cast metal framework	\$520
D5730	Reline complete maxillary denture, chairside	\$95
D6010	Surgical placement of implant body	Not covered
D6240	Pontic, porcelain fused to high noble metal	\$395
D7140	Extraction, erupted tooth or exposed root	\$20
D7210	Surgical extraction of erupted tooth	\$75
D7240	Removal of impacted tooth, completely bony	\$175
D7953	Bone replacement graft for ridge preservation	Not covered
D8080	Comprehensive orthodontic treatment, adolescent	\$1,800
D8090	Comprehensive orthodontic treatment, adult	\$2,100
D8680	Orthodontic retention, removal of appliances and retainers	\$275
D9110	Palliative treatment of dental pain, minor procedure	\$25
D9223	Deep sedation or general anesthesia, per 15 minutes	\$65
D9944	Occlusal guard, hard appliance, full arch	\$185

7.3 DHMO-Specific Rules

- **Specialty referrals.** The PGD must submit a referral request to ClassicHealth before the member sees an endodontist, periodontist, oral surgeon, orthodontist or pediatric dentist. Standard referral decisions are issued within 5 business days. Once approved, the specialist copay schedule applies; specialist copays run approximately 25% higher than the general dentist copays shown above.

- **Orthodontic case fee.** The listed orthodontic copay covers up to 24 months of active treatment plus 24 months of retention. Treatment extending beyond 24 months is billed by the orthodontist at \$175 per additional quarter. Records, extractions and appliances not included in comprehensive treatment are billed separately at the schedule rate.
- **Broken appointments.** The PGD may charge up to \$25 for an appointment cancelled with less than 24 hours' notice.
- **Out-of-area emergency.** A member more than 50 miles from the assigned PGD may receive palliative emergency treatment from any licensed dentist and submit the itemized bill for reimbursement up to \$75 per incident. Definitive treatment must be completed by the PGD.
- **Lab fees.** Copays shown include all laboratory charges for standard materials. Upgraded materials (for example, a precious-metal crown when a base-metal crown is the schedule standard) are billed as the difference in lab cost, disclosed in writing before treatment.

8. ClassicHealth VisionPlus PPO (CH-VIS-PPO-2026)

8.1 Plan Design at a Glance

VisionPlus PPO covers a routine eye examination and either eyeglasses or contact lenses. Members may use any provider; in-network use produces materially lower out-of-pocket cost.

Provision	In-Network	Out-of-Network
Deductible	None	None
Exam copay	\$10	Reimbursed up to \$45
Materials copay (lenses and frames combined)	\$25	Applied against the reimbursement schedule
Exam frequency	Once every 12 months	Once every 12 months
Lens frequency	Once every 12 months	Once every 12 months
Frame frequency	Once every 24 months	Once every 24 months
Contact lens frequency (in lieu of glasses)	Once every 12 months	Once every 12 months
Frame allowance	\$180, plus 20% off any amount over the allowance	Reimbursed up to \$70
Elective contact lens allowance	\$180 (includes the fitting and evaluation)	Reimbursed up to \$105
Network size	39,800 access points including independent optometrists, ophthalmologists and national retail chains	—

8.2 Covered Materials and Lens Options

Table 8-A. VisionPlus PPO materials benefit. In-network amounts are member cost after the \$25 materials copay.

Item	In-Network member cost	Out-of-Network reimbursement
Single vision lenses	\$0 after materials copay	Up to \$40
Lined bifocal lenses	\$0 after materials copay	Up to \$60
Lined trifocal lenses	\$0 after materials copay	Up to \$80
Lenticular lenses	\$0 after materials copay	Up to \$125
Standard progressive lenses	\$65	Up to \$60
Premium progressive lenses	\$65 plus 20% off the retail balance	Up to \$60
Polycarbonate lenses — dependent children under 19	\$0	Included in the lens allowance
Polycarbonate lenses — adults	\$35	Included in the lens allowance
Standard anti-reflective coating	\$45	Not separately reimbursed
Premium anti-reflective coating	\$68	Not separately reimbursed
Photochromic (transition) lenses	\$75	Not separately reimbursed
Scratch-resistant coating	\$17	Not separately reimbursed

Item	In-Network member cost	Out-of-Network reimbursement
Ultraviolet coating	\$16	Not separately reimbursed
Tint, solid or gradient	\$17	Not separately reimbursed
High-index lenses (1.67 or greater)	\$80	Not separately reimbursed
Frames	\$180 allowance, then 20% off the balance	Up to \$70
Elective contact lenses and fitting	\$180 allowance	Up to \$105
Medically necessary contact lenses	\$0 after the materials copay, prior authorization required	Up to \$210
Safety eyewear (industrial)	Not covered under this plan	Not covered

Medically necessary contact lenses are covered in full after the materials copay when prior authorization documents keratoconus, anisometropia of 3.00 diopters or more, high ametropia over ± 10.00 diopters, irregular corneal astigmatism, aphakia, or a corrected visual acuity that improves to at least 20/70 with contacts when it cannot be corrected to that level with spectacles.

8.3 Additional VisionPlus Features

- **Second pair discount.** 40% off a complete second pair of prescription eyeglasses purchased from the same in-network provider within 12 months of the covered pair.
- **Non-covered items discount.** 20% off lens options and frames purchased outside of the frequency schedule; 20% off non-prescription sunglasses; 15% off contact lens purchases beyond the allowance.
- **Laser vision correction.** 15% off the retail price or 5% off the promotional price at contracted LASIK and PRK centers. This is a discount, not an insured benefit.
- **Diabetic Eyecare Program.** Members with diabetes receive an additional retinal examination each plan year at a \$20 copay, plus follow-up care for diabetic retinopathy, glaucoma and macular degeneration at the specialist cost share.
- **Low vision benefit.** One comprehensive low vision evaluation every 60 months at \$0, and up to \$1,000 every 24 months toward prescribed low vision aids, at 75% coinsurance, with prior authorization.
- **Medical eye conditions.** Treatment of eye disease or injury — conjunctivitis, glaucoma, cataract surgery, retinal detachment, foreign body removal — is a medical benefit and is covered under the Premier PPO or Select HMO, not the vision plan.

9. ClassicHealth VisionCare HMO (CH-VIS-HMO-2026)

9.1 Plan Design at a Glance

VisionCare HMO is a closed-network vision plan with lower premiums and no out-of-network benefit. Members must use a VisionCare Select provider.

Provision	Value
Deductible	None
Exam copay	\$0
Materials copay	\$15
Exam frequency	Once every 12 months
Lens frequency	Once every 12 months
Frame frequency	Once every 24 months
Contact lens frequency	Once every 12 months, in lieu of glasses
Frame benefit	Any frame in the VisionCare Select Collection at \$0, or a \$130 allowance toward any other frame with no discount on the balance
Contact lens benefit	\$130 allowance including fitting and evaluation
Out-of-network benefit	None. Claims from non-participating providers are denied in full.
Provider change	Not required in advance; members may use any VisionCare Select provider

9.2 Covered Materials — VisionCare HMO

Item	Member cost
Comprehensive eye examination with dilation	\$0
Contact lens fitting and evaluation, standard	\$0 (applied against the \$130 allowance)
Single vision lenses	\$0 after the \$15 materials copay
Lined bifocal lenses	\$0 after the \$15 materials copay
Lined trifocal lenses	\$0 after the \$15 materials copay
Standard progressive lenses	\$55
Polycarbonate lenses — dependent children under 19	\$0
Polycarbonate lenses — adults	\$30
Standard anti-reflective coating	\$40
Photochromic lenses	\$70
Scratch coating	\$15
Tint	\$15
High-index lenses	\$70
Frames from the Select Collection	\$0

Item	Member cost
Frames outside the Select Collection	\$130 allowance; member pays 100% of the balance
Elective contact lenses	\$130 allowance; member pays the balance
Medically necessary contact lenses	\$0 after the materials copay, prior authorization required

9.3 VisionCare HMO Rules

- The Select Collection contains approximately 180 frame styles refreshed twice a year and is stocked at every participating location.
- Because there is no out-of-network benefit, a member who buys glasses online or from a non-participating retailer receives no reimbursement.
- The plan pays for either eyeglasses or contact lenses in a benefit period, never both. Choosing contacts consumes both the lens and the frame benefit.
- Diabetic retinal screening is covered at \$0 once per plan year in addition to the routine exam.
- Broken or lost eyewear is not replaced outside the frequency schedule; a 20% courtesy discount applies to replacement purchases at participating locations.
- Members enrolled in VisionCare HMO may not also enroll in VisionPlus PPO.

10. Prior Authorization and Utilization Management

10.1 Who Is Responsible

For **in-network** services, the participating provider is contractually responsible for obtaining prior authorization. If an in-network provider fails to obtain it and the service is later found not medically necessary, the member is held harmless and the provider absorbs the cost.

For **out-of-network** services on the Premier PPO, the **member** is responsible. Failure to obtain prior authorization results in a **\$500 penalty per occurrence**, applied before benefits are calculated. The penalty does not count toward the deductible or the out-of-pocket maximum. If the service is later determined not to have been medically necessary, no benefits are payable.

10.2 Services Requiring Prior Authorization

Table 10-A. Prior authorization requirements by plan. "Yes" indicates authorization must be obtained before the service is rendered.

Service category	Premier PPO	Select HMO	Notes
All elective inpatient admissions	Yes	Yes	Notify at least 5 business days before admission
Emergency inpatient admissions	Notification within 48 hours	Notification within 48 hours	No authorization required; notification only
Observation stays exceeding 48 hours	Yes	Yes	Converted to inpatient review
Skilled nursing facility	Yes	Yes	Concurrent review every 7 days
Inpatient rehabilitation	Yes	Yes	Concurrent review every 7 days
Long-term acute care	Yes	Yes	—
Home health care	Yes	Yes	Initial 12 visits, then reauthorization
Home infusion therapy	Yes	Yes	—
Hospice	Notification	Notification	—
Advanced imaging: MRI, MRA, CT, CTA, PET, nuclear cardiology	Yes	Yes	Administered through the ClassicHealth Radiology Management Program
Genetic and genomic testing	Yes	Yes	Excludes routine prenatal screening and newborn screening
Sleep studies, attended in-facility	Yes	Yes	Home studies do not require authorization
Durable medical equipment over \$1,000 purchase price or \$250 monthly rental	Yes	Yes	Includes CPAP, wheelchairs, hospital beds, prosthetic limbs
Bariatric surgery	Yes	Yes	See criteria in Section 10.4
Spinal surgery, including fusion and laminectomy	Yes	Yes	Conservative therapy documentation required
Total joint replacement (hip, knee, shoulder)	Yes	Yes	Site-of-service review applies
Transplant evaluation and transplant	Yes	Yes	Center of Excellence required

Service category	Premier PPO	Select HMO	Notes
Cardiac catheterization, elective	Yes	Yes	—
Implantable devices (pacemaker, defibrillator, neurostimulator)	Yes	Yes	—
Cosmetic or potentially cosmetic procedures (blepharoplasty, panniculectomy, rhinoplasty, breast reduction, vein procedures)	Yes	Yes	Functional impairment documentation required
Gender-affirming surgical procedures	Yes	Yes	WPATH-aligned criteria
Applied behavior analysis	Yes	Yes	Reauthorization every 6 months
Transcranial magnetic stimulation and electroconvulsive therapy	Yes	Yes	—
Residential treatment and partial hospitalization, behavioral health	Yes	Yes	Concurrent review every 7 days
Hyperbaric oxygen therapy	Yes	Yes	—
Proton beam therapy and stereotactic radiosurgery	Yes	Yes	—
Specialty drugs (medical or pharmacy benefit)	Yes	Yes	See Section 5.3
Provider-administered infusion drugs over \$2,000 per administration	Yes	Yes	Site-of-care redirection may apply
Out-of-network services (PPO only)	Yes	Not applicable	\$500 penalty if omitted
Non-emergency air ambulance	Yes	Yes	—
Clinical trial participation	Notification	Notification	Routine patient costs covered
Fertility treatment (PPO only)	Yes	Not applicable	IVF lifetime maximum applies

10.3 Decision Timeframes

Request type	Decision issued within	Notice to member and provider
Standard pre-service (medical)	5 business days, not to exceed 15 calendar days	Written notice within 1 business day of the decision
Expedited pre-service (urgent)	72 hours	Verbal notice immediately, written within 3 calendar days
Concurrent review (continued stay)	24 hours for urgent continued care	Verbal at the time of the decision
Post-service (retrospective)	30 calendar days	Written
Standard pharmacy prior authorization	72 hours	Electronic and written
Expedited pharmacy prior authorization	24 hours	Verbal and written
Formulary exception, standard	72 hours	Written

Request type	Decision issued within	Notice to member and provider
Formulary exception, expedited	24 hours	Verbal and written
Additional information requested	The clock pauses up to 14 calendar days for the member or provider to respond	Written request stating exactly what is needed

If ClassicHealth fails to issue a decision within the applicable timeframe, the request is **deemed approved** and the service is covered as if authorized.

10.4 Selected Medical Necessity Criteria

ClassicHealth applies MCG Health care guidelines as the primary criteria set, supplemented by ASAM criteria for substance use disorder levels of care, and internally developed policies where no national guideline exists. All criteria are available to members and providers on request at no charge, and are posted at providers.classichealth.example.com/policies.

Bariatric surgery. Requires all of the following: age 18 or older; BMI of 40 or greater, or BMI of 35 or greater with at least one qualifying comorbidity (type 2 diabetes, obstructive sleep apnea, hypertension, non-alcoholic steatohepatitis, or degenerative joint disease); documented participation in a physician-supervised weight management program for at least 6 consecutive months within the preceding 24 months; a psychological evaluation clearing the member for surgery; nutritional evaluation; and tobacco cessation for at least 8 weeks before surgery. Surgery must be performed at a facility accredited by the MBSAQIP. One revision is covered per lifetime for a documented surgical complication or inadequate weight loss due to an anatomic problem.

Spinal fusion, lumbar. Requires documented failure of at least 6 weeks of conservative therapy including physical therapy and analgesics, imaging correlating with the clinical findings, and one of: spondylolisthesis with instability, degenerative disc disease with severe functional limitation, recurrent disc herniation at the same level, spinal stenosis with neurogenic claudication, or trauma or tumor.

Advanced imaging. Requires that the clinical question cannot be answered by a less costly modality, that the study will change management, and that the study is not a duplicate of one performed in the prior 90 days for the same indication without a change in clinical status.

Applied behavior analysis. Requires a diagnosis of autism spectrum disorder made by a licensed psychologist, developmental pediatrician, neurologist or psychiatrist using standardized instruments; a treatment plan with measurable goals authored by a Board Certified Behavior Analyst; and documented progress at each 6-month reauthorization.

Gender-affirming surgery. Requires persistent, well-documented gender dysphoria; capacity to consent; age of majority in the state of residence; reasonable control of any significant medical or mental health concerns; and, for genital surgery, 12 months of continuous hormone therapy and 12 months of living in a gender role congruent with gender identity unless hormones are clinically contraindicated.

10.5 Programs Members Are Enrolled In

Program	Who is eligible	What it provides	Cost to member
24/7 Nurse Line	All members	Registered nurse triage, symptom guidance, care-setting recommendations	\$0

Program	Who is eligible	What it provides	Cost to member
Complex Case Management	Members with catastrophic or multi-system conditions, identified by predictive model or referral	An assigned RN case manager, care plan coordination, benefit exception review	\$0, voluntary
Chronic Condition Management	Diabetes, asthma, COPD, coronary artery disease, congestive heart failure, chronic kidney disease	Coaching calls, remote monitoring devices where indicated, gap-in-care outreach	\$0, voluntary
Maternity Program (ClassicBeginnings)	Pregnant members who enroll by the end of the first trimester	Risk screening, nurse support, a \$150 wellness incentive at postpartum visit completion	\$0
Behavioral Health Navigation	Members seeking mental health or substance use care	Appointment scheduling within 7 days for routine, 48 hours for urgent	\$0
Second Opinion Service	Members facing surgery, a cancer diagnosis, or a rare disease	Expert review by a specialist at a national academic center	\$0
Centers of Excellence	Transplant, bariatric surgery, joint replacement, spine surgery, cardiac surgery, oncology	Contracted high-volume facilities; travel and lodging allowance of up to \$5,000 per episode when the nearest COE is more than 100 miles from home	\$0 travel benefit
Tobacco Cessation	All members age 18+	Coaching, all FDA-approved cessation medications at \$0	\$0

11. Claims Submission and Payment

11.1 How Claims Are Filed

In-network providers submit claims directly. A member normally files a claim only for out-of-network services on the PPO plans, for international care, or for services paid up front.

Item	Medical	Dental	Vision
Member claim form	CH-CLM-100	CH-DCLM-200 (or an ADA 2024 claim form)	CH-VCLM-300
Paper claims address	ClassicHealth Claims, PO Box 91240, Chicago, IL 60691-0240	ClassicHealth Dental Claims, PO Box 91250, Chicago, IL 60691-0250	ClassicHealth Vision Claims, PO Box 91260, Chicago, IL 60691-0260
EDI payer ID	58217	58218	58219
Electronic claim standard	ANSI X12N 837P and 837I, version 5010	ANSI X12N 837D	ANSI X12N 837P
Provider claim filing deadline (contracted)	90 days from the date of service	90 days	90 days
Member claim filing deadline	12 months from the date of service	12 months	12 months
Claim status	my.classichealth.example.com or 1-800-555-0142	1-800-555-0163	1-800-555-0171

A member claim must include: the itemized bill showing the provider's name, address, tax identification number and NPI; the date and place of service; the CPT, HCPCS or ADA procedure code; the ICD-10 diagnosis code; the charge for each service; the member ID and group number; and proof of payment if reimbursement is sought.

11.2 Payment Timeframes and Interest

Claim type	Payment or denial issued within
Clean electronic claim	30 calendar days of receipt
Clean paper claim	45 calendar days of receipt
Claim requiring additional information	30 days from receipt of the last item requested; the request must be made within 30 days of the original claim
Emergency services claim	30 calendar days
Pharmacy point-of-sale	Adjudicated in real time

Interest accrues on clean claims not paid within the applicable period at the statutory rate of the member's state of residence, with a floor of 9% per annum in Illinois. Interest under \$5.00 is not paid separately but is aggregated quarterly.

11.3 Explanation of Benefits

An Explanation of Benefits (EOB) is issued for every processed claim. It is not a bill. Key fields:

EOB field	Meaning
Billed amount	What the provider charged
Allowed amount	The maximum the Plan recognizes for the service
Provider discount	Billed amount minus allowed amount; the member never owes this on in-network claims
Deductible applied	The portion of the allowed amount credited to the member's deductible
Copay	Fixed dollar amount owed
Coinsurance	Percentage of the allowed amount owed by the member
Plan paid	What ClassicHealth paid the provider
Member responsibility	Deductible plus copay plus coinsurance plus any non-covered charges
Remark codes	Explain reductions, denials and adjustments; a legend is printed on the reverse
Not covered	Charges the Plan does not cover; the member may owe these unless the provider is contractually barred from billing

11.4 Common Denial Reason Codes

Code	Description	What the member should do
CH-101	Service not covered under the plan	Review Section 15; if the member believes the service is covered, file an appeal
CH-102	Prior authorization not obtained	Provider may request a retrospective review within 60 days with clinical documentation
CH-104	Not medically necessary	Adverse benefit determination; appeal rights apply (Section 12)
CH-108	Experimental or investigational	Appeal, then external review; the external reviewer must include a physician with relevant expertise
CH-112	Out-of-network provider, HMO plan	Only emergency, urgent, and authorized care is payable
CH-115	Frequency or benefit limit exceeded	Compare against the frequency tables in Sections 3.3, 4.3, 6.3
CH-118	Duplicate claim	No action needed
CH-121	Member not eligible on the date of service	Verify enrollment with the employer's benefits administrator
CH-124	Coordination of benefits information needed	Complete the annual COB questionnaire in the member portal
CH-127	Filing limit exceeded	Provider write-off if contracted; member may appeal with proof of timely filing
CH-130	Bundled into another service	Provider-level adjustment; member is not billed
CH-133	Alternate benefit applied (dental)	Member owes the difference between the treatment provided and the least expensive alternative
CH-136	Work-related injury; workers' compensation is primary	File with the workers' compensation carrier
CH-139	Third-party liability under investigation	Complete the accident questionnaire within 30 days to avoid a hold

11.5 Overpayments and Recoveries

ClassicHealth may recover an overpayment from a provider within **24 months** of the payment date, or within 36 months where fraud or intentional misrepresentation is involved. Written notice states the member, claim number, date of service, amount and reason. The provider has 30 days to dispute before offset against future payments begins. Member overpayments are requested in writing and are never offset against future benefits without the member's consent, except where required by the group policy.

12. Appeals, Grievances and External Review

12.1 Definitions

- **Adverse benefit determination** — a denial, reduction, termination, or failure to pay for a benefit, including a rescission of coverage and a determination that a service is experimental, investigational, or not medically necessary.
- **Appeal** — a request to reconsider an adverse benefit determination.
- **Grievance** — a complaint about anything other than an adverse benefit determination: provider access, quality of service, wait times, staff conduct, or plan administration.

12.2 Internal Appeals

Stage	Who may file	Filing deadline	Decision deadline	Reviewer
Level 1 — pre-service	Member or authorized representative, including the treating provider with the member's consent	180 days from the adverse determination	30 calendar days	A clinician who was not involved in the original decision and does not report to the original decision-maker
Level 1 — post-service	Same	180 days	60 calendar days	Same
Level 1 — urgent / expedited	Same; a provider may request expedited review without member consent	Any time	72 hours	Same
Level 2 — voluntary second level	Member	60 days from the Level 1 decision	30 calendar days pre-service, 45 days post-service	Appeals panel including at least one physician in the same or a similar specialty
Concurrent care termination	Member	Before the reduction takes effect	24 hours	Same

Every appeal decision letter states the specific reason for the decision, the plan provisions relied on, the clinical criteria applied (or an offer to provide them free of charge), the member's right to receive all documents relevant to the claim free of charge, and the next level of review available including external review and, for ERISA plans, the right to bring a civil action under section 502(a).

12.3 External Review

An independent review organization (IRO) accredited by URAC and randomly assigned from a rotating panel decides external reviews. ClassicHealth pays the full cost; the member pays nothing.

Item	Standard external review	Expedited external review
When available	After internal appeals are exhausted, or if ClassicHealth fails to follow the internal process	Immediately, when delay would seriously jeopardize life, health, or the ability to regain maximum function
Filing deadline	4 months from the final internal adverse determination	Any time
Preliminary eligibility review	5 business days	Immediately

Item	Standard external review	Expedited external review
Documents forwarded to the IRO	Within 5 business days	Within 24 hours
Additional member information accepted	Within 10 business days of the IRO assignment	As available
Decision issued	45 calendar days from receipt of the request	72 hours
Binding on the Plan	Yes	Yes
Eligible issues	Medical necessity, appropriateness, health care setting, level of care, effectiveness, experimental or investigational determinations, and rescission of coverage	Same

A decision by the IRO in the member's favor is binding on ClassicHealth, which must provide coverage immediately, even if it disagrees.

12.4 Grievances

Grievances may be filed by phone at 1-800-555-0142, in the member portal, or in writing to ClassicHealth Grievances, PO Box 91280, Chicago, IL 60691-0280.

- Acknowledged in writing within **5 business days**.
- Resolved and communicated within **30 calendar days**.
- Quality-of-care grievances are referred to the Chief Medical Officer's quality management committee, and the outcome of a peer review is confidential under state peer review statutes; the member is told that the review was completed.
- Filing a grievance never affects a member's coverage or their relationship with their providers.

12.5 Authorized Representatives

A member may appoint anyone — a family member, an attorney, or the treating provider — as an authorized representative by completing form CH-AR-410. For urgent care claims, a treating provider may act as the representative without a signed form. An authorized representative appointment remains valid for one year or until revoked in writing.

12.6 Regulator Contacts

A member who is dissatisfied after exhausting the Plan's process may contact the insurance regulator in their state. For Illinois-issued policies: Illinois Department of Insurance, Office of Consumer Health Insurance, 320 West Washington Street, Springfield, IL 62767, 1-877-527-9431. For ERISA plans, the Employee Benefits Security Administration of the U.S. Department of Labor may be reached at 1-866-444-3272.

13. Coordination of Benefits, Subrogation and Medicare

13.1 Order of Benefit Determination

When a member is covered by more than one plan, ClassicHealth follows the NAIC model coordination of benefits rules. Total payments from all plans never exceed 100% of the allowed expense.

1. **Non-dependent vs. dependent.** The plan covering the person as an employee, member or subscriber pays before the plan covering that person as a dependent.
2. **Dependent child, parents married or living together.** The **birthday rule** applies: the plan of the parent whose birthday falls earlier in the calendar year (month and day, not year) is primary. If both parents have the same birthday, the plan in force longer is primary.
3. **Dependent child, parents separated or divorced.** A court decree naming the financially responsible parent controls. Absent a decree: the custodial parent's plan, then the custodial parent's spouse's plan, then the non-custodial parent's plan, then the non-custodial parent's spouse's plan. If the decree gives joint custody without naming responsibility, the birthday rule applies.
4. **Active vs. inactive employee.** The plan covering the person as an active employee is primary over a plan covering that person as a laid-off or retired employee, or as a dependent of one.
5. **COBRA continuation.** The plan covering the person as an employee or dependent of an employee is primary over COBRA continuation coverage.
6. **Longer / shorter length of coverage.** If none of the above resolves the order, the plan that covered the person longer is primary.

ClassicHealth uses the **maintenance of benefits** method as the secondary payer: the secondary payment is the lesser of (a) the amount ClassicHealth would have paid as primary, minus what the primary plan paid, or (b) the member's remaining out-of-pocket liability. The Plan is not required to pay more than it would have paid as the primary plan.

13.2 Subrogation and Right of Reimbursement

When a third party is or may be responsible for an injury or illness — a motor vehicle accident, a slip and fall, a defective product, an assault — ClassicHealth has the right to recover what it paid.

- The member must notify ClassicHealth within **30 days** of filing any claim or lawsuit against a third party, and must complete the accident questionnaire (form CH-TPL-600).
- The member must cooperate, must not do anything that prejudices the Plan's rights, and must hold any recovery in trust for the Plan to the extent of benefits paid.
- The Plan's right applies to the **first dollars** of any settlement or judgment. The Plan does not apply the "make whole" doctrine or the common fund doctrine unless required by the law governing the policy.
- The Plan will consider a proportional reduction for attorney's fees on a case-by-case basis at the discretion of the Recovery Unit.
- Failure to cooperate permits the Plan to deny or suspend future claims related to the injury and to offset against future benefits.

Recovery Unit contact: ClassicHealth Recovery Services, PO Box 91290, Chicago, IL 60691-0290, 1-800-555-0195.

13.3 Medicare Coordination

Situation	Primary payer
Member is age 65+ and actively working for an employer with 20 or more employees	ClassicHealth group plan
Member is age 65+, retired, or the employer has fewer than 20 employees	Medicare
Member is disabled and under 65, employer has 100 or more employees	ClassicHealth group plan
Member is disabled and under 65, employer has fewer than 100 employees	Medicare
Member has end-stage renal disease	ClassicHealth group plan for the first 30 months of Medicare ESRD eligibility, then Medicare
Member is covered under COBRA and is entitled to Medicare	Medicare

All ClassicHealth medical plans provide **creditable prescription drug coverage**, meaning coverage is expected to pay out at least as much as standard Medicare Part D. Members who keep creditable coverage will not incur a Part D late enrollment penalty if they later enroll. A written creditable coverage notice is mailed each year before October 15.

13.4 Workers' Compensation and Motor Vehicle Coverage

Work-related injury and illness is excluded from all ClassicHealth plans; the workers' compensation carrier is responsible. If a claim is disputed, ClassicHealth may pay conditionally and pursue reimbursement from the workers' compensation carrier. In no-fault automobile states, medical payments coverage and personal injury protection are primary to the ClassicHealth plan up to the automobile policy's limits.

14. Continuation of Coverage

14.1 COBRA

Employers with 20 or more employees must offer COBRA continuation. ClassicHealth or the group's COBRA administrator sends the election notice.

Qualifying event	Who may continue	Maximum continuation period
Termination of employment (other than for gross misconduct)	Employee, spouse, dependent children	18 months
Reduction of hours below the eligibility threshold	Employee, spouse, dependent children	18 months
Determination of Social Security disability within the first 60 days of continuation	The disabled qualified beneficiary and family	29 months total
Death of the employee	Spouse, dependent children	36 months
Divorce or legal separation	Former spouse, dependent children	36 months
Dependent child ceasing to be eligible	The child	36 months
Employee becoming entitled to Medicare, followed by termination or reduction of hours	Spouse, dependent children	36 months from the Medicare entitlement date

Key deadlines:

- The employer must notify the plan administrator within **30 days** of a termination, reduction of hours, death, or Medicare entitlement.
- The member must notify the plan administrator within **60 days** of a divorce, legal separation, or a child losing dependent status.
- The election notice must be furnished within **14 days** of the administrator's receipt of notice.
- The qualified beneficiary has **60 days** from the later of the notice date or the loss-of-coverage date to elect.
- The first premium payment is due within **45 days** of the election. Subsequent premiums are due on the first of the month with a **30-day grace period**.
- Premium is 102% of the full group cost (employer plus employee share), or **150%** during months 19 through 29 of a disability extension.
- Coverage elected is retroactive to the date coverage would otherwise have ended, so there is no gap.

COBRA ends early if: premium is not paid on time; the qualified beneficiary becomes covered under another group plan; the qualified beneficiary becomes entitled to Medicare after electing COBRA; the employer stops offering any group health plan; or a disability determination is reversed.

14.2 State Continuation ("Mini-COBRA")

Employees of groups with fewer than 20 employees may be eligible for state continuation. Terms vary by state; Illinois provides 12 months of continuation for employees covered for at least 3 months, with a dependent spousal continuation of up to 2 years, and up to 5 years or to age 65 for a spouse age 55 or older. Contact Member Services for the rule that applies to a specific policy.

14.3 Other Continuation Rights

Situation	Right
Military leave (USERRA)	Up to 24 months of continuation; no waiting period or exclusion on reinstatement
FMLA leave	Coverage continues on the same terms for up to 12 weeks (26 weeks for military caregiver leave); the employee must continue paying their share
Leave of absence approved by the employer	Per the group policy, typically to the end of the third month
Layoff with recall rights	Per the group policy
Retiree coverage	Only where the employer has purchased a retiree rider
Surviving dependents	36 months of COBRA, or a state continuation right
Conversion to an individual policy	ClassicHealth does not offer individual conversion policies; members are referred to the ACA Marketplace, where loss of group coverage is a special enrollment event with a 60-day window

15. General Exclusions and Limitations

The following are not covered under any ClassicHealth medical plan. Plan-specific dental and vision exclusions are in Sections 6.5, 8.3 and 9.3. Where a state mandate requires coverage of an item listed here, the mandate controls for policies issued in that state.

15.1 Categorical Exclusions

- **Cosmetic services** — procedures performed primarily to improve appearance, without restoring function. Reconstructive surgery following an accident, an infection, a congenital anomaly in a dependent child, or a mastectomy is covered.
- **Experimental or investigational services** — items not approved by the FDA for the condition being treated, not supported by peer-reviewed literature or nationally recognized compendia, or under Phase I or II clinical study. Routine patient costs in an approved Phase III or IV clinical trial for a life-threatening condition are covered.
- **Custodial care** — help with activities of daily living that does not require the skill of licensed medical personnel, whether provided at home, in an assisted living facility, or in a nursing home.
- **Long-term care and residential room and board** for non-medical reasons, including assisted living, group homes, wilderness programs, therapeutic boarding schools, and halfway houses.
- **Services with no charge** or that would be free absent insurance, and services provided by a member's immediate family, by the member themselves, or by an employer's on-site clinic where no charge is made.
- **Care covered by another program** — workers' compensation, government programs, Veterans Affairs care for a service-connected condition, and care required by a court that is not otherwise medically necessary.
- **Non-emergency care outside the United States** — emergency and urgent care abroad is covered; planned or elective treatment, including medical tourism, is not.
- **Services rendered before the effective date or after the termination date** of coverage.
- **Charges above the allowed amount** from out-of-network providers, except where the No Surprises Act applies.
- **Missed appointment fees, completion of forms, copying of records, and interest or late fees** charged by a provider.

15.2 Service-Specific Exclusions

Excluded item	Detail and any exception
Weight loss programs, gym memberships, exercise equipment	Bariatric surgery and physician-supervised obesity counseling are covered; commercial weight loss programs are not. Anti-obesity medications are covered only under the terms of the pharmacy benefit with prior authorization.
Routine foot care	Trimming of nails, corns, calluses. Covered when the member has diabetes with peripheral neuropathy or peripheral vascular disease.
Orthopedic shoes and arch supports	Covered when part of a leg brace, or for diabetic members meeting therapeutic shoe criteria.
Private duty nursing	Not covered in any setting. Skilled home health visits are covered as described in Sections 3.2 and 4.2.
Personal comfort items	Televisions, telephones, air conditioners, humidifiers, whirlpool baths, exercise devices.
Reversal of sterilization	Not covered on either plan.
Erectile dysfunction and sexual dysfunction treatment	Medications and devices are not covered; the diagnostic evaluation of an underlying medical condition is covered.

Excluded item	Detail and any exception
Surrogacy	Services for a person acting as a surrogate for another are not covered by the surrogate's ClassicHealth plan; the Plan asserts a right of reimbursement against the intended parents.
Cryopreservation and storage of embryos, sperm or oocytes	Not covered. Fertility preservation before gonadotoxic cancer therapy is covered on the PPO within the IVF lifetime maximum.
Genetic testing for non-medical purposes	Ancestry, paternity, and direct-to-consumer testing.
Dental care under the medical plan	Excluded except for accidental injury to sound natural teeth within 12 months of the accident, oral tumors and cysts, orthognathic surgery for a functional deformity, and anesthesia for dental care in a member under 7 or with a developmental disability.
Vision correction surgery	LASIK, PRK, radial keratotomy and refractive lens exchange are excluded; a discount is available through VisionPlus (Section 8.3).
Hearing aids beyond the stated limit	The plans cover \$2,500 per ear every 36 months; batteries, cords and repairs beyond the warranty are excluded.
Alternative therapies	Massage therapy, naturopathy, homeopathy, hypnotherapy, aromatherapy, biofeedback for non-covered indications. Acupuncture is covered on the PPO only, up to 12 visits.
Travel, lodging and transportation	Excluded except for the Centers of Excellence travel allowance and covered ambulance transport.
Educational and vocational services	Special education, vocational rehabilitation and job training. Applied behavior analysis is a covered medical benefit, not an educational service.
Nutritional supplements and formula	Excluded except for enteral formula delivered by tube, and metabolic formula for inborn errors of metabolism such as PKU.
Growth hormone for idiopathic short stature	Excluded; covered for documented growth hormone deficiency, Turner syndrome, Prader-Willi syndrome, chronic kidney disease and SGA without catch-up growth.
Charges for a hospital stay that begins on a Friday or Saturday for surgery scheduled the following Monday	Not covered unless medically necessary or the surgery is performed within 24 hours of admission.

16. Glossary of Terms

Term	Definition
Allowed amount	The maximum amount the Plan recognizes as payable for a covered service. Also called the eligible expense, negotiated rate, or maximum reimbursable charge.
Ambulatory surgery center (ASC)	A licensed facility where surgery is performed on an outpatient basis; generally a lower-cost site of service than a hospital outpatient department.
Appeal	A request to reconsider an adverse benefit determination.
Balance billing	A bill from an out-of-network provider for the difference between the provider's charge and the Plan's allowed amount. Prohibited for in-network providers and for the situations covered by the No Surprises Act.
Benefit period	The 12-month span, aligned to the calendar year for all ClassicHealth 2026 plans, over which deductibles, out-of-pocket maxima and frequency limits are measured.
Capitation	A fixed monthly payment per enrolled member paid to a provider regardless of the services used. Used in the DentalFirst DHMO and for some Select HMO primary care arrangements.
Centers of Excellence (COE)	Facilities contracted for high-volume, high-complexity procedures with demonstrated outcome advantages.
Coinsurance	The member's percentage share of the allowed amount after the deductible has been met.
Concurrent review	Utilization review conducted while care is being delivered, typically during an inpatient stay.
Coordination of benefits (COB)	Rules that determine the payment order when a member has more than one plan.
Copayment	A fixed dollar amount a member pays for a covered service.
Cost sharing	The general term for deductibles, copayments and coinsurance. Premium is not cost sharing.
Creditable coverage	Prescription drug coverage expected to pay, on average, at least as much as standard Medicare Part D.
Deductible	The amount a member pays each plan year before the Plan begins paying coinsurance.
Durable medical equipment (DME)	Equipment that can withstand repeated use, is used for a medical purpose, and is appropriate for use in the home.
Embedded deductible	A family deductible structure in which each individual has their own deductible cap within the family amount.
Evidence of Coverage (EOC)	The legal document describing benefits, exclusions and member rights for a specific plan.
Explanation of Benefits (EOB)	A statement showing how a claim was processed. Not a bill.
Formulary	The list of prescription drugs the Plan covers, organized by tier.
Grievance	A complaint about something other than a benefit denial.
Health Maintenance Organization (HMO)	A plan that requires the use of a defined network and, generally, a primary care physician who coordinates and authorizes care.
In-network	A provider under contract with ClassicHealth who accepts the allowed amount as payment in full.
Independent review organization (IRO)	An accredited, independent entity that decides external appeals.
Least expensive alternate treatment (LEAT)	A dental provision under which the Plan pays toward the least costly professionally acceptable treatment.

Term	Definition
Maintenance of benefits	A COB method under which the secondary plan pays no more than it would have paid as primary, minus what the primary plan paid.
Medically necessary	See Section 1.5.
No Surprises Act	Federal law effective 2022 limiting balance billing for emergency care, air ambulance, and non-emergency care by out-of-network providers at in-network facilities.
Out-of-pocket maximum	The most a member pays for covered in-network services in a plan year, after which the Plan pays 100%.
Preferred Provider Organization (PPO)	A plan that pays for care from any licensed provider but pays more when the member uses network providers, with no referral requirement.
Prior authorization	Approval that must be obtained before certain services are rendered.
Primary care physician (PCP)	The physician who provides and coordinates a member's care under the HMO.
Prudent layperson standard	The emergency coverage standard based on what a reasonable non-medical person would believe about their symptoms.
Qualifying life event (QLE)	An event permitting an enrollment change outside of open enrollment.
Retrospective review	Utilization review performed after care has been delivered.
Step therapy	A requirement to try a preferred drug before a non-preferred drug is covered.
Subrogation	The Plan's right to recover benefits paid from a third party responsible for the member's injury.
Summary of Benefits and Coverage (SBC)	A standardized federal disclosure of plan benefits and costs, in a uniform format for comparison.
Tier (network)	A cost-sharing level assigned to providers based on contract terms and quality and efficiency measures.
Tier (pharmacy)	A cost-sharing level assigned to a drug on the formulary.
Usual, customary and reasonable (UCR)	A charge basis for out-of-network dental services, set at the 90th percentile of charges for the same service in the same geographic area.
Utilization management (UM)	The set of processes — prior authorization, concurrent review, retrospective review — used to evaluate medical necessity and appropriateness.

17. Premium Rates and Employer Contributions

17.1 Standard Monthly Rates, Plan Year 2026

Table 17-A. Composite monthly premium by plan and tier. Rates shown are the ClassicHealth standard book rates for a group of 100–499 enrolled employees in the Chicago rating area. Actual group rates vary by region, group size, industry, and experience.

Plan	Employee only	Employee + spouse	Employee + child(ren)	Family
ClassicHealth Premier PPO (medical)	\$612.40	\$1,298.30	\$1,164.60	\$1,836.20
ClassicHealth Select HMO (medical)	\$528.90	\$1,120.25	\$1,004.90	\$1,585.60
ClassicHealth DentalChoice PPO	\$42.80	\$85.60	\$92.15	\$138.40
ClassicHealth DentalFirst DHMO	\$22.15	\$43.10	\$48.90	\$70.25
ClassicHealth VisionPlus PPO	\$9.85	\$18.70	\$19.95	\$28.60
ClassicHealth VisionCare HMO	\$6.40	\$12.20	\$13.05	\$18.75

17.2 Standard Employer Contribution Model

The ClassicHealth standard funding arrangement, which a group may vary, is:

Coverage tier	Employer share	Employee share
Employee only	75%	25%
Dependent portion (spouse, children, family)	55%	45%

Worked example — an employee electing Premier PPO family coverage, DentalChoice PPO family, and VisionPlus PPO family under the standard model:

Line	Monthly amount
Premier PPO family premium	\$1,836.20
Employee-only portion of that premium	\$612.40
Employer pays 75% of the employee-only portion	\$459.30
Dependent portion (\$1,836.20 – \$612.40)	\$1,223.80
Employer pays 55% of the dependent portion	\$673.09
Employer medical contribution	\$1,132.39
Employee medical contribution	\$703.81
Employee dental contribution (DentalChoice family, same formula)	\$53.72
Employee vision contribution (VisionPlus family, same formula)	\$10.90
Total employee monthly payroll deduction	\$768.43

17.3 Rating Rules

- Rates are guaranteed for 12 months from the group's effective date unless enrollment changes by more than 10%, the benefit design changes, or a change in law materially affects the cost of coverage.
- Premium is due on the first day of the coverage month, with a **31-day grace period**. Coverage terminates retroactively to the last day of the paid period if premium is not received by the end of the grace period.
- Groups with 51 or more employees are experience-rated using a 24-month claims lookback with pooling of individual claims above \$175,000.
- Groups with 50 or fewer employees are community-rated on the applicable state small-group rating tables, with variation permitted only by age, geography, tobacco use and family size.
- Premiums may be paid pre-tax through a Section 125 cafeteria plan where the employer sponsors one.
- A wellness premium differential of up to 30% of the employee-only cost (50% for tobacco cessation programs) is available where the employer has purchased the ClassicWell rider.

18. Compliance Notices and Member Rights

18.1 Nondiscrimination

ClassicHealth complies with applicable federal civil rights law and Section 1557 of the Affordable Care Act. It does not discriminate on the basis of race, color, national origin, age, disability, sex, sexual orientation or gender identity. Free aids and services are provided to people with disabilities and free language services to people whose primary language is not English. Civil rights grievances may be filed with the ClassicHealth Civil Rights Coordinator at 1400 Meridian Tower, 220 North Wacker Drive, Chicago, IL 60606, 1-800-555-0142, or with the U.S. Department of Health and Human Services Office for Civil Rights.

18.2 The No Surprises Act

Members are protected from balance billing in three situations: emergency services from an out-of-network provider or facility; air ambulance services from an out-of-network provider; and non-emergency services from an out-of-network provider at an in-network facility, such as an anesthesiologist, radiologist, pathologist, assistant surgeon, hospitalist or intensivist the member did not choose.

In these situations the member owes only the in-network cost share, that amount counts toward the in-network deductible and out-of-pocket maximum, and the provider may not bill the balance. A member may voluntarily waive these protections for certain non-emergency services only by signing a written notice-and-consent form at least 72 hours before the appointment; a member is never required to sign, and consent may not be requested for emergency services, air ambulance, or ancillary services such as anesthesiology and radiology. Good faith estimates are available for scheduled services on request. Questions or suspected violations: 1-800-985-3059 (federal No Surprises Help Desk).

18.3 Mental Health Parity (MHPAEA)

Financial requirements and treatment limitations applied to mental health and substance use disorder benefits are no more restrictive than those applied to substantially all medical and surgical benefits in the same classification. This applies to quantitative limits such as copays, deductibles and visit limits, and to non-quantitative limits such as prior authorization, step therapy, network admission standards, and provider reimbursement methods. ClassicHealth's comparative analysis of non-quantitative treatment limitations is available free of charge on request.

18.4 Women's Health and Cancer Rights Act (WHCRA)

A member receiving benefits for a mastectomy who elects breast reconstruction is entitled to coverage of: reconstruction of the breast on which the mastectomy was performed; surgery and reconstruction of the other breast to produce a symmetrical appearance; prostheses; and treatment of physical complications at all stages of the mastectomy, including lymphedema. Coverage is subject to the same deductible and coinsurance as other benefits under the plan.

18.5 Newborns' and Mothers' Health Protection Act

Group health plans generally may not restrict benefits for any hospital stay in connection with childbirth to less than **48 hours following a vaginal delivery or 96 hours following a cesarean section**. The Plan may not require prior authorization for these stays. The attending provider and the mother may agree to an earlier discharge.

18.6 HIPAA Privacy and Security

ClassicHealth maintains administrative, physical and technical safeguards for protected health information (PHI). PHI is used and disclosed for treatment, payment and health care operations without authorization; other uses require the member's written authorization. Members have the right to inspect and copy their records, request an amendment, receive an accounting of disclosures, request confidential communications, request restrictions on use, and receive a copy of the Notice of Privacy Practices. Notices of a breach of unsecured PHI are provided without unreasonable delay and within 60 days. Privacy Office: 1-800-555-0193, privacy@classichealth.example.com.

18.7 ERISA Rights (for plans subject to ERISA)

Participants are entitled to examine, without charge, all documents governing the plan, and to obtain copies on written request for a reasonable copying charge. Participants are entitled to receive a summary annual report. Plan fiduciaries have a duty to act prudently and in the interest of participants and beneficiaries. No one may fire a participant or otherwise discriminate against them to prevent them from obtaining a benefit or exercising ERISA rights. If a claim is denied in whole or in part, the participant has the right to a written explanation, to review documents, and to appeal. After exhausting the plan's appeals, a participant may file suit in federal court.

18.8 Transparency in Coverage

ClassicHealth publishes machine-readable files of in-network negotiated rates and out-of-network allowed amounts, updated monthly, at classichealth.example.com/transparency. Members may use the online cost estimator in the member portal to obtain a personalized estimate of cost sharing for 500+ shoppable services before receiving care, or request the same information by phone at 1-800-555-0142.

18.9 Other Required Notices

Notice	Substance
Michelle's Law	A dependent child on medically necessary leave of absence from post-secondary education retains eligibility for up to 12 months.
Genetic Information Nondiscrimination Act (GINA)	The Plan does not use genetic information for eligibility, premium setting or underwriting, and does not request or require genetic testing.
Uniform Modification / SBC	The Summary of Benefits and Coverage is provided at enrollment, at renewal, and within 7 business days of any request, free of charge, in a uniform format. Material modifications require 60 days' advance notice.
Medicaid and CHIP premium assistance	Members in certain states may be eligible for premium assistance to help pay for employer coverage; contact the state Medicaid agency or 1-877-KIDS-NOW.
Prescription drug creditable coverage	Notice mailed annually before October 15 to Medicare-eligible members.
Continuity of care (CAA)	Members in a continuing course of treatment with a provider who leaves the network may elect up to 90 days of transitional in-network care.
Provider directory accuracy	Directories are verified at least every 90 days. If a member relied on an inaccurate directory listing, the member pays no more than the in-network cost share.

19. Contact Directory

Purpose	Contact	Hours (Central Time)
Member Services — medical, eligibility, ID cards, claims	1-800-555-0142	Mon–Fri 7:00 a.m.–8:00 p.m.; Sat 8:00 a.m.–2:00 p.m.
24/7 Nurse Line	1-800-555-0177	24 hours, every day
Behavioral Health and Substance Use Navigation	1-800-555-0188	24 hours, every day
Pharmacy Services	1-800-555-0155	Mon–Fri 7:00 a.m.–9:00 p.m.; Sat–Sun 8:00 a.m.–5:00 p.m.
Meridian Specialty Rx	1-800-555-0159	Mon–Fri 7:00 a.m.–8:00 p.m.
Dental Member Services	1-800-555-0163	Mon–Fri 7:00 a.m.–7:00 p.m.
Vision Member Services	1-800-555-0171	Mon–Fri 7:00 a.m.–7:00 p.m.
Prior Authorization intake	1-800-555-0181, fax 1-312-555-0198	Mon–Fri 7:00 a.m.–7:00 p.m.; after-hours voicemail for urgent requests
Appeals and Grievances	1-800-555-0142, fax 1-312-555-0199	Mon–Fri 8:00 a.m.–5:00 p.m.
Recovery Services (subrogation, COB)	1-800-555-0195	Mon–Fri 8:00 a.m.–5:00 p.m.
Privacy Office	1-800-555-0193	Mon–Fri 8:00 a.m.–5:00 p.m.
Fraud, Waste and Abuse hotline	1-800-555-0197 (anonymous reporting accepted)	24 hours
Provider Services	1-800-555-0149	Mon–Fri 7:00 a.m.–7:00 p.m.
Employer Group Services	1-800-555-0145	Mon–Fri 8:00 a.m.–6:00 p.m.
TTY / TDD	711	Same as the line being reached
Language assistance	1-800-555-0142, select option 9	Same as Member Services

Mailing purpose	Address
Medical claims	ClassicHealth Claims, PO Box 91240, Chicago, IL 60691-0240
Dental claims	ClassicHealth Dental Claims, PO Box 91250, Chicago, IL 60691-0250
Vision claims	ClassicHealth Vision Claims, PO Box 91260, Chicago, IL 60691-0260
Appeals	ClassicHealth Appeals Unit, PO Box 91270, Chicago, IL 60691-0270
Grievances	ClassicHealth Grievances, PO Box 91280, Chicago, IL 60691-0280
Subrogation and recovery	ClassicHealth Recovery Services, PO Box 91290, Chicago, IL 60691-0290
Corporate correspondence	ClassicHealth Insurance Company, 1400 Meridian Tower, 220 North Wacker Drive, Chicago, IL 60606

20. Frequently Asked Questions

1. What is the difference between the Premier PPO and the Select HMO? The PPO lets a member see any provider without a referral and covers out-of-network care at 50% after a higher deductible. The HMO costs less in premium and cost sharing but requires a primary care physician, requires referrals for specialists, and covers no out-of-network care except emergencies, urgent care and pre-authorized exceptions.

2. How much is my medical deductible? Premier PPO: \$1,500 per individual and \$3,000 per family in Tier 1, \$2,250 / \$4,500 in Tier 2, and \$3,000 / \$6,000 out of network. Select HMO: \$500 per individual and \$1,000 per family.

3. What is my out-of-pocket maximum? Premier PPO Tier 1: \$5,000 individual and \$10,000 family. Select HMO: \$3,500 individual and \$7,000 family. Prescription drug cost sharing counts toward these amounts on both plans.

4. Do I need a referral to see a specialist? On the Premier PPO, no. On the Select HMO, yes, except for OB-GYN care, behavioral health, family planning, STI treatment, routine eye exams and emergency or urgent care.

5. What does an office visit cost? Premier PPO: \$30 for primary care and \$50 for a specialist, with the deductible waived. Select HMO: \$20 for primary care and \$40 for a specialist, deductible waived.

6. How much is an emergency room visit? Premier PPO: a \$350 copay plus 20% coinsurance after the deductible; the copay is waived if the member is admitted. Select HMO: a \$300 copay, waived if admitted. Emergency care is always paid at the in-network level, anywhere in the world.

7. Is telehealth covered? Yes. Virtual primary care through ClassicHealth Virtual Care is \$0 on both plans. Virtual urgent care is \$15 on the PPO and \$10 on the HMO.

8. What do prescriptions cost? Premier PPO 30-day retail: \$10 generic, \$40 preferred brand, \$75 non-preferred brand after a \$150 pharmacy deductible, and 25% coinsurance capped at \$250 for specialty. Select HMO: \$5, \$30, \$60, and 20% capped at \$200, with no pharmacy deductible. Insulin is capped at \$35 per 30-day supply on both plans.

9. My drug was moved to a higher tier in the middle of the year. What can I do? ClassicHealth gives 60 days' advance notice for mid-year formulary changes. A member already taking the drug can request continuity of therapy and stay at the old cost share through December 31. A prescriber can also submit a formulary exception, which is decided in 72 hours (24 hours if expedited).

10. Do I need prior authorization? For in-network care, the provider is responsible for obtaining it. For out-of-network care on the PPO, the member is responsible, and skipping it costs a \$500 penalty that does not count toward the deductible or out-of-pocket maximum. The full list is in Section 10.2.

11. How long does a prior authorization decision take? Standard medical requests: 5 business days, never more than 15 calendar days. Urgent requests: 72 hours. Pharmacy: 72 hours standard and 24 hours expedited. If the Plan misses the deadline, the request is deemed approved.

12. My annual physical was supposed to be free but I got a bill. Why? If the visit also addressed a new or ongoing problem — a medication adjustment, a new symptom — the provider may bill a separate diagnostic office visit alongside the preventive service. The preventive portion stays at \$0 and the diagnostic portion is subject to the office visit copay.

13. My screening colonoscopy generated a bill. Is that right? A screening colonoscopy is \$0 in network, including polyp removal and pathology during the screening, and including a follow-up colonoscopy after a positive stool-based test. A colonoscopy ordered to investigate symptoms is diagnostic, not preventive, and is subject to the deductible and coinsurance. If a screening was coded as diagnostic, ask the provider to review the coding, then

appeal if needed.

14. Are my kids covered until 26? Yes, to the end of the month in which the child turns 26, regardless of marital status, student status, residence, financial dependence, or the availability of other coverage. A child disabled before 26 may continue indefinitely with certification on form CH-DD-220.

15. I just had a baby. What do I need to do? The newborn is automatically covered for 31 days. To keep coverage, enroll the child within 30 days of birth. Coverage is retroactive to the date of birth.

16. I'm getting married. When can I add my spouse? Within 30 days of the wedding. Coverage begins the first of the month following the request.

17. What happens to my coverage if I leave my job? Coverage ends on the last day of that month. A COBRA election notice arrives within 14 days of the plan administrator being notified; a member has 60 days to elect and 45 days after electing to make the first payment. COBRA coverage is retroactive, so there is no gap.

18. How much does COBRA cost? 102% of the full group premium — the employer's share plus the employee's share — or 150% during months 19 through 29 of a Social Security disability extension.

19. How do I change my primary care physician on the HMO? Through the member portal or by calling 1-800-555-0142. A change requested by the 20th of a month takes effect the first of the following month.

20. What if I need care while traveling? Emergency and urgent care are covered anywhere at the in-network level, including outside the United States. Planned or elective care abroad is not covered. For international claims, pay the provider, then submit form CH-INTL-500 with an itemized bill and proof of payment within 12 months.

21. My dentist says I need a crown. What will I pay? On the DentalChoice PPO, a crown is a Class III service: the Plan pays 50% of the allowed amount after the \$50 deductible, subject to the \$2,000 annual maximum, and one crown per tooth every 60 months. On the DentalFirst DHMO, a porcelain-fused-to-metal crown is a flat \$395 copay with no deductible and no annual maximum. Ask for a pre-treatment estimate on the PPO if the treatment plan exceeds \$500.

22. Why did my dental plan pay less than my dentist charged for a white filling on a back tooth? The alternate benefit provision. The Plan pays the amalgam allowance for a posterior filling and the member pays the difference if a composite is placed.

23. How many cleanings do I get? Two per plan year on both dental plans. On the DentalChoice PPO, members who are pregnant, have diabetes, have periodontal disease, or have had a heart valve replacement may qualify for a third and fourth cleaning at 100% through the Enhanced Benefit; call 1-800-555-0163 to enroll.

24. Is orthodontia covered? DentalChoice PPO: 50% up to a \$2,000 lifetime maximum for dependent children under 19 at the start of treatment, after a 12-month waiting period. DentalFirst DHMO: a flat case copay of \$1,800 for adolescents and \$2,100 for adults, covering 24 months of active treatment plus retention.

25. Are dental implants covered? On the DentalChoice PPO, yes, as a Class III service at 50%, subject to the annual maximum and the missing tooth clause. On the DentalFirst DHMO, implants are not covered.

26. What does my vision plan pay for glasses? VisionPlus PPO: a \$10 exam copay, a \$25 materials copay, a \$180 frame allowance every 24 months with 20% off the balance, and lenses every 12 months. VisionCare HMO: a \$0 exam, a \$15 materials copay, and either a Select Collection frame at \$0 or a \$130 allowance with no discount on the balance.

27. Can I get contacts and glasses in the same year? No. The plans cover either lenses and a frame, or contact lenses, in a benefit period — not both.

28. Is LASIK covered? No. Refractive surgery is excluded on all plans. VisionPlus members receive a 15% discount at contracted laser centers.

29. Who pays first when my spouse and I both have coverage? The plan covering a person as an employee pays before the plan covering them as a dependent. For children with married parents, the birthday rule applies: whichever parent's birthday falls earlier in the calendar year has the primary plan.

30. I was in a car accident. What happens? Notify ClassicHealth within 30 days and complete the accident questionnaire (form CH-TPL-600). In no-fault states, the auto policy's medical payments coverage pays first. ClassicHealth has a right of reimbursement from any settlement.

31. My claim was denied. How do I appeal? File a Level 1 appeal within 180 days of the denial. Pre-service appeals are decided in 30 days, post-service in 60 days, and urgent appeals in 72 hours. A voluntary Level 2 appeal is available, and after that an independent external review that is binding on the Plan and free to the member.

32. Can my doctor appeal for me? Yes, with the member's written consent on form CH-AR-410. For urgent claims, the treating provider may act as the authorized representative without a signed form.

33. Where do I find out what something will cost before I get it? Use the cost estimator in the member portal, which covers more than 500 shoppable services, or call Member Services. Providers must also furnish a good faith estimate on request under the No Surprises Act.

34. What is a Center of Excellence and do I have to use one? A contracted high-volume facility for transplants, bariatric surgery, joint replacement, spine surgery, cardiac surgery and oncology. Transplants must use a Center of Excellence on both plans. For other procedures it is optional but comes with a travel and lodging allowance of up to \$5,000 when the nearest center is more than 100 miles away.

35. Is my coverage creditable for Medicare Part D? Yes. Both medical plans provide creditable prescription drug coverage, so a member who keeps this coverage and enrolls in Part D later will not owe a late enrollment penalty. A written notice is mailed each year before October 15.

21. Appendix A — Quick Comparison Matrix

Table A-1. Side-by-side summary of all six 2026 plans.

Feature	Premier PPO	Select HMO	DentalChoice PPO	DentalFirst DHMO	VisionPlus PPO	VisionCare HMO
Plan ID	CH-MED-PP O-2026	CH-MED-HMO-2026	CH-DEN-PP O-2026	CH-DEN-HMO-2026	CH-VIS-PP O-2026	CH-VIS-HMO-2026
Individual deductible	\$1,500 T1	\$500	\$50	None	None	None
Family deductible	\$3,000 T1	\$1,000	\$150	None	None	None
Out-of-pocket max, individual	\$5,000 T1	\$3,500	Not applicable	Not applicable	Not applicable	Not applicable
Annual benefit maximum	None	None	\$2,000	None	Frequency-based	Frequency-based
Referral required	No	Yes	No	Yes, for specialists	No	No
Out-of-network coverage	Yes, 50%	Emergency and urgent only	Yes, reduced	Emergency only, \$75	Yes, schedule	None
Primary care / provider selection	Optional	PCP required	Optional	PGD required	Optional	Optional within network
Preventive care	\$0	\$0	100% Class I	\$0 exams and cleanings	\$10 exam	\$0 exam
Employee-only monthly premium	\$612.40	\$528.90	\$42.80	\$22.15	\$9.85	\$6.40
Family monthly premium	\$1,836.20	\$1,585.60	\$138.40	\$70.25	\$28.60	\$18.75

22. Appendix B — Forms Index

Form number	Title	When it is used
CH-ENR-010	Group Enrollment and Change Form	New hire enrollment, open enrollment, qualifying life events
CH-DP-100	Affidavit of Domestic Partnership	Enrolling a domestic partner
CH-DD-220	Disabled Dependent Certification	Continuing a disabled child past age 26
CH-TOC-310	Transition of Care Request	Continuing treatment with a non-participating provider
CH-CLM-100	Medical Claim Form	Member-submitted medical claims
CH-DCLM-200	Dental Claim Form	Member-submitted dental claims
CH-VCLM-300	Vision Claim Form	Out-of-network vision reimbursement
CH-INTL-500	International Claim Form	Emergency or urgent care received outside the United States
CH-AR-410	Authorized Representative Designation	Appointing someone to act on the member's behalf
CH-APP-420	Appeal Request Form	Level 1 and Level 2 internal appeals
CH-EXT-430	External Review Request	Independent external review
CH-TPL-600	Accident and Third-Party Liability Questionnaire	Any injury that may involve a third party
CH-COB-610	Coordination of Benefits Questionnaire	Annual COB verification, or when other coverage is identified
CH-PA-700	Prior Authorization Request (medical)	Provider-submitted authorization requests
CH-PA-710	Pharmacy Prior Authorization Request	Prescriber-submitted drug authorization
CH-FE-720	Formulary Exception Request	Requesting coverage of a non-formulary drug
CH-PRIV-800	Authorization to Release Protected Health Information	Disclosing PHI to a third party
CH-PRIV-810	Request for Confidential Communications	Restricting how the Plan contacts a member
CH-DHMO-SCHED-2026	DentalFirst Copayment Schedule	Full 420-code DHMO copay listing

23. Appendix C — Retrieval and Indexing Notes

This appendix supports downstream indexing of this document by an automated retrieval system. It is not part of the plan terms.

23.1 Canonical Entities

Entity type	Canonical values
Carrier	ClassicHealth Insurance Company
Plan year	2026 (January 1 – December 31)
Plan IDs	CH-MED-PPO-2026, CH-MED-HMO-2026, CH-DEN-PPO-2026, CH-DEN-HMO-2026, CH-VIS-PPO-2026, CH-VIS-HMO-2026
Product lines	medical, dental, vision
Structures	PPO, HMO, DHMO
Network tiers	Tier 1 Preferred, Tier 2 Extended, Out-of-Network, Select Network, DentalChoice Network, DentalFirst Network, VisionPlus Network, VisionCare Select Network
Cost-share types	deductible, copay, coinsurance, out-of-pocket maximum, allowance, annual maximum, lifetime maximum
Document types represented	plan design, benefit grid, limitations, exclusions, utilization management, claims, appeals, coordination of benefits, continuation, rates, compliance notices, contacts, FAQ

23.2 Section-to-Intent Map

Member intent	Authoritative section
"What is my deductible / out-of-pocket max?"	3.1, 4.1, 6.1, Appendix A
"How much for a visit, ER, imaging, surgery?"	3.2, 4.2
"How many visits do I get?"	3.3, 4.3, 6.3
"Is preventive care free?"	3.4
"What does my drug cost / is it covered?"	5.2, 5.3, 5.4
"Do I need a referral or prior authorization?"	4.4, 10.1, 10.2
"How long will approval take?"	10.3
"What does my dental plan pay?"	6.2, 6.3, 7.2
"What does my vision plan pay?"	8.1, 8.2, 9.1, 9.2
"How do I file a claim?"	11.1, 11.2
"Why was my claim denied?"	11.4
"How do I appeal?"	12.1–12.5
"Who pays first?"	13.1, 13.3
"I'm leaving my job / getting divorced"	2.4, 2.5, 14.1
"Is this covered?"	15.1, 15.2, 6.5

Member intent	Authoritative section
"What does this term mean?"	16
"What does it cost per month?"	17.1, 17.2
"Who do I call?"	19

23.3 Known Ambiguities to Disambiguate Before Answering

An automated assistant answering from this document should confirm the following before giving a number, because the answer differs by plan:

1. **Which product line** — medical, dental or vision. "What is my deductible?" has three different answers.
2. **PPO or HMO** — cost sharing differs materially on every line.
3. **Network tier** — Premier PPO has three tiers; quoting Tier 1 when the member used Tier 2 understates cost by 10 percentage points of coinsurance.
4. **In-network or out-of-network** — VisionCare HMO and DentalFirst DHMO have no out-of-network benefit at all.
5. **Adult or dependent child** — orthodontia, polycarbonate lenses, sealants and fluoride are age-limited.
6. **Preventive or diagnostic** — the same CPT code can be \$0 or subject to a copay depending on the reason for the visit.
7. **Retail, mail order or specialty** — pharmacy cost sharing differs by channel as well as by tier.
8. **Emergency or non-emergency** — out-of-network emergency care is paid at the in-network level on both medical plans.

End of document. ClassicHealth 2026 Master Plan Reference Guide, document ID CH-MPRG-2026-v1.0. This document describes fictional insurance products created for demonstration and system-development purposes. It is not an offer of insurance and does not describe any real insurance product, company, or coverage.